

UK Legal & Compliance Pack

Practice-management platform for therapists · Faresay as data processor, therapist as controller

DRAFT for professional sign-off. These are working drafts prepared to support a UK launch. They are not legal advice and must be reviewed and finalised by a qualified solicitor before reliance. Placeholders marked [PLACEHOLDER] require completion.

11 documents

Client & customer terms

1. Privacy Policy
2. Terms of Service
3. Therapist Agreement

Data protection (processor model)

4. Data Processing Agreement (UK GDPR Art 28)
5. Technical & Organisational Measures (Annex 2)
6. Sub-processor List (Annex 3)
7. Record of Processing Activities (Art 30)
8. Data Protection Impact Assessment (Art 35)

Operational policies

9. Security & Data Protection Policy
10. Clinical Governance Policy
11. Crisis & Safeguarding Policy

Privacy Policy

⚠️ DRAFT v0.1 — for legal/privacy counsel review. NOT legal advice. Counsel must finalise before use. Last updated: 26 June 2026

Faresay Privacy Policy

Faresay operates an online therapy / mental-health marketplace that connects clients with licensed mental-health professionals. This Privacy Policy explains how we collect, use, share and protect your personal information, and the rights you have over it.

Because Faresay is a mental-health service, much of the information we handle is **special-category / sensitive health data**. We treat this information with a correspondingly high standard of confidentiality and security. Please read this policy carefully.

This policy should be read alongside our [Terms of Service](#), our [Security & Data Protection Policy](#), and (where applicable) the separate clinician-provided privacy notices and any HIPAA Notice of Privacy Practices (see Section 16).

1. Who we are & data controller ⚠️ COUNSEL

Faresay ("Faresay", "we", "us", "our") provides the technology platform through which clients discover, book and pay for sessions with independent licensed mental-health professionals ("therapists" or "clinicians").

- **Controller legal entity:** Faresay Ltd
- **Registered address:** 167-169 Great Portland Street, London W1W 5PF
- **Company registration number:** 17302034
- **ICO registration number (UK):** applied for 26 June 2026 — registration number to follow
- **Data Protection Officer / privacy contact:** Joel Harvey, Founder — support@faresay.com

⚠️ **COUNSEL — controller vs processor / joint-controller mapping.** Faresay provides the platform; the clinician provides the clinical service and owns the clinical relationship and clinical record (see [CONTEXT.md] / [Therapist Agreement](#)). The respective data-protection roles must be confirmed: - For **account, marketplace, payment, device and usage data**, Faresay is likely the **controller**. - For **clinical / session-note data created by the clinician**, the clinician may be the **independent controller** (with Faresay acting as **processor**, or as **joint controller** for certain activities), and in the US the clinician may be the **covered entity** with Faresay a **business associate**. - Counsel must finalise the controller / processor / joint-controller analysis (UK/EU) and the covered-entity / business-associate analysis (US HIPAA), and ensure the resulting roles are reflected consistently across this policy, the Terms, the Therapist Agreement and any Business Associate Agreements (BAAs) and Data Processing Agreements (DPAs).

2. Scope (UK + US)

This policy currently applies to Faresay's services as offered in the **United Kingdom**, and is being prepared to also cover Faresay's planned expansion into the **United States (all 50 states + DC)**.

- **United Kingdom (now):** governed by the **UK GDPR** and the **Data Protection Act 2018**.
- **United States (planned expansion):** governed by **HIPAA** (where applicable) and applicable **US state privacy and consumer-health-data laws** (e.g. California **CCPA/CPRA**, Washington **My Health My Data Act**, and other state comprehensive privacy laws).

⚠ COUNSEL — Until the US service launches, US-specific provisions in this policy are forward-looking and must be confirmed and finalised before any US-facing version is published. Some jurisdictions may require a separate or supplemented notice.

This policy covers our website, web and mobile applications, and related services (together, the "Platform"). It does **not** cover third-party websites or services we link to, which have their own privacy practices.

3. What data we collect

We collect the following categories of personal information.

3.1 Account & identity data

- Name, email address, telephone number, username and password (stored hashed).
- Date of birth / age confirmation.
- Country, state/region and (where relevant) address or location.
- Profile information you provide.
- For clinicians: professional identity, licence/registration details, qualifications, jurisdictions of licensure, and verification information.

3.2 Health & special-category data **⚠ COUNSEL**

This is the most sensitive information we handle. It may include: - The fact that you are seeking or receiving mental-health support (itself sensitive health data). - Intake / assessment information, presenting concerns, symptoms, history and goals. - Session-related clinical information and notes (typically created and held by your clinician — see Section 1). - Messages and communications relating to your care. - Any health, disability, or other special-category information you choose to share.

Under UK GDPR this is **special-category data** (Article 9). In the US it is likely **protected health information (PHI)** under HIPAA and/or **consumer health data** under state laws such as the Washington My Health My Data Act. We apply a high confidentiality and security bar to all such data.

⚠ COUNSEL — confirm the precise boundary between data Faresay processes and clinical-record data the clinician controls, and ensure consent / authorisation mechanisms meet both UK GDPR Article 9 and US HIPAA / state consumer-health-data requirements (including, where required, separate valid consent for collection, use and sharing of consumer health data).

3.3 Payment data

- Billing details and transaction history.
- Payment is processed by third-party payment providers; we do **not** store full card numbers on our own systems. [PLACEHOLDER: confirm exactly what payment data Faresay stores vs. what the payment processor stores.]

3.4 Device & usage data

- IP address, device identifiers, browser type, operating system.
- Log data, pages/screens viewed, features used, and timestamps.
- Approximate location derived from IP address.
- Cookies and similar technologies (see Section 11).

3.5 Communications data

- Messages you send to us (support, enquiries) and, where applicable, in-platform messages with clinicians.
- Records of your communications with our support team.
- Feedback, reviews and survey responses.

We collect this information directly from you, automatically through your use of the Platform, and in some cases from third parties (e.g. clinicians regarding your bookings, identity/licence verification providers regarding clinicians, and payment providers regarding transactions).

4. How and why we use your data, with legal bases

The table below summarises our main purposes and the **legal bases** we rely on under UK GDPR. For health / special-category data we identify both an **Article 6** basis and an **Article 9** condition.

 **COUNSEL** — the legal bases below are a first draft and must be confirmed. In particular, the Article 9 condition for health data and the US legal basis (HIPAA treatment/payment/healthcare-operations and any required authorisations, plus state-law consent requirements) must be finalised by counsel.

#	Purpose	Data used	UK GDPR Art 6 basis	UK GDPR Art 9 condition (health data)
1	Create and manage your account	Account/identity	Contract (Art 6(1)(b))	n/a
2	Match you with clinicians and enable booking/scheduling	Account/identity, limited health context	Contract (Art 6(1)(b))	 COUNSEL — likely Art 9(2)(h) (health/social care) or explicit consent Art 9(2)(a)
3	Facilitate the provision of mental-health care via clinicians	Health & special-category	Contract / Legitimate interests	 COUNSEL — Art 9(2)(h) provision of health care, and/or Art 9(2)(a) explicit consent
4	Process payments and operate the 15% platform fee	Payment, account	Contract (Art 6(1)(b))	n/a
5	Provide support and respond to enquiries	Communications, account	Contract / Legitimate interests	Art 9(2)(a) / 9(2)(h) where health data is involved  COUNSEL
6	Safety, safeguarding and crisis response	Health, communications	Legitimate interests / Legal	 COUNSEL — Art 9(2)(c) vital interests and/or 9(2)

#	Purpose	Data used	UK GDPR Art 6 basis	UK GDPR Art 9 condition (health data)
			obligation / Vital interests	(h); see Crisis & Safeguarding Policy
7	Security, fraud prevention, and protecting the Platform	Device/usage, account	Legitimate interests / Legal obligation	n/a
8	Service improvement and analytics	Device/usage (minimised/aggregated where possible)	Legitimate interests	⚠️ COUNSEL — avoid using health data for analytics without an appropriate condition
9	Marketing communications (where permitted)	Account, usage	Consent / Legitimate interests	n/a
10	Legal compliance, regulatory and dispute handling	As needed	Legal obligation / Legitimate interests	Art 9(2)(f) legal claims; 9(2)(g)/(h) as applicable ⚠️ COUNSEL

US legal basis ⚠️ **COUNSEL.** Where HIPAA applies, uses and disclosures of PHI will be made for **treatment, payment and healthcare operations** and otherwise as **permitted or required by HIPAA**, with patient authorisation obtained where required. State consumer-health-data laws (e.g. Washington MHMDA) may require **separate, specific consent** for the collection and sharing of consumer health data. Counsel must confirm the HIPAA basis, Faresay's covered-entity vs business-associate status, and the required state-law consents before US launch.

We will not use your special-category health data for a new, incompatible purpose without identifying an appropriate lawful basis and, where required, obtaining your explicit consent.

5. How we share your data

We share personal information only as described below.

- **Therapists / clinicians.** To enable and deliver your care, relevant information is shared with the clinician you book or are matched with. The clinician is an independent professional responsible for the clinical relationship and clinical record (see Section 1).
- **Service providers / processors & sub-processors.** We use third-party providers to host, operate and support the Platform (e.g. cloud hosting, communications, analytics, identity/licence verification, customer support). They process data on our instructions under written contracts (DPAs, and BAAs where HIPAA applies). [PLACEHOLDER: sub-processor list / link to sub-processor list.]
- **Payment providers.** Payment processors handle transactions and operate under their own privacy notices. [PLACEHOLDER: name(s) of payment provider(s).]
- **Legal & safety.** We may disclose information where necessary to comply with law, respond to lawful requests, enforce our Terms, prevent fraud, or protect the rights, safety and vital interests of you or others (including in safeguarding / crisis situations — see [Crisis & Safeguarding Policy](#)).
- **Business transfers.** If Faresay is involved in a merger, acquisition, financing or sale of assets, personal information may be transferred as part of that transaction, subject to appropriate confidentiality and continued

protection of your data.

We do **not** sell your personal information. ⚠️ **COUNSEL** — confirm "sale"/"share" definitions under CCPA/CPRA and "sale" of consumer health data under MHMDA, and ensure our disclosures and any required opt-out mechanisms match actual data flows (including any analytics/advertising trackers).

6. International data transfers ⚠️ **COUNSEL**

Faresay is based in the UK and plans to operate in the US, and some of our service providers may process data outside the UK/EEA (including in the US). Where we transfer personal data internationally, we will put in place an appropriate safeguard, which may include:

- **UK International Data Transfer Agreement (IDTA)** or the **UK Addendum** to the EU Standard Contractual Clauses (**SCCs**);
- **EU Standard Contractual Clauses** for EEA-origin transfers;
- reliance on an **adequacy** decision / the **UK Extension to the EU–US Data Privacy Framework**, where applicable; or
- other lawful transfer mechanisms.

⚠️ **COUNSEL** — finalise the transfer mechanism(s) for each data flow (UK/EU → US in particular), confirm current adequacy / Data Privacy Framework status, and complete transfer risk assessments (TRAs / TIAs) where required. Given the sensitivity of mental-health data, transfer safeguards warrant particular scrutiny.

7. Data retention ⚠️ **COUNSEL**

We keep personal information only for as long as necessary for the purposes set out in this policy, and to meet legal, regulatory, accounting, clinical-record and dispute-resolution requirements.

- Account & identity data: [PLACEHOLDER: retention period]
- Health & clinical-record data: [PLACEHOLDER: retention period] ⚠️ **COUNSEL** — clinical-record retention is governed by professional/clinical standards and law (and may differ between UK and US, and by state). Confirm who (Faresay vs clinician) is responsible for retaining clinical records and for how long.
- Payment & transaction data: [PLACEHOLDER: retention period, typically driven by tax/accounting law]
- Device, usage & log data: [PLACEHOLDER: retention period]
- Communications & support data: [PLACEHOLDER: retention period]

When data is no longer required, we will securely delete or anonymise it. ⚠️ **COUNSEL** — finalise the full retention schedule and reconcile it with clinical-record obligations and US state law.

8. Security

We implement technical and organisational measures designed to protect personal information against unauthorised access, loss, misuse or alteration, with enhanced controls for sensitive health data. These measures are described in our **Security & Data Protection Policy**, which this policy incorporates by reference.

No system can be guaranteed 100% secure, but we maintain a high security bar appropriate to the sensitivity of mental-health data.

9. Your rights

9.1 UK / EU data-subject rights

Subject to applicable law, you have the right to: - **access** the personal data we hold about you; - request **rectification** of inaccurate data; - request **erasure** ("right to be forgotten") in certain circumstances; - **restrict** or **object** to certain processing; - **data portability**; - **withdraw consent** at any time where processing is based on consent (without affecting prior processing); and - not be subject to solely automated decisions with legal/similarly significant effects (see Section 13).

To exercise these rights, contact us using the details in Section 17. We will respond within the timeframes required by law. ⚠️ **COUNSEL** — note that some rights (e.g. erasure) may be limited where data forms part of a clinical record or must be retained for legal/regulatory reasons.

9.2 US state rights ⚠️ **COUNSEL**

Depending on your state of residence and applicable law, you may have rights including to **know/access, delete, correct, opt out of sale/share/targeted advertising, data portability**, and to **non-discrimination** for exercising your rights.

- **California (CCPA/CPRA)**: rights to know, delete, correct, opt out of sale/sharing, and limit use of sensitive personal information.
- **Washington My Health My Data Act**: specific **consumer-health-data** rights, including the right to **withdraw consent**, to **access**, and to **delete** consumer health data, and protections requiring consent for collection and sharing.
- **Other state comprehensive privacy laws**: [PLACEHOLDER: list applicable states and rights as US footprint is confirmed].

⚠️ **COUNSEL** — finalise the full list of US state rights, the request/verification and appeal mechanisms, response timeframes, authorised-agent handling, and any required "Your Privacy Choices" / opt-out links before US launch. Where HIPAA applies, individual rights over PHI are governed by HIPAA and the separate Notice of Privacy Practices (Section 16).

10. Cookies & tracking

We use cookies and similar technologies to operate the Platform, remember your preferences, keep you signed in, maintain security, and understand and improve how the Platform is used.

- **Strictly necessary** cookies are required for the Platform to function.
- **Analytics / performance** and **functional** cookies are used [PLACEHOLDER: confirm categories and providers].
- We will obtain consent for non-essential cookies where required (e.g. under PECR / UK law), and provide controls to manage your preferences.

⚠️ **COUNSEL** — given the sensitivity of mental-health data, scrutinise any third-party trackers/SDKs (especially advertising/analytics) for potential disclosure of health-related data; certain trackers on a health service may trigger HIPAA and state consumer-health-data restrictions. Confirm the cookie banner/consent mechanism. [PLACEHOLDER: link to detailed cookie notice / preference centre.]

11. Children's data

The Platform is intended for adults. [PLACEHOLDER: confirm minimum age — e.g. 18, or 16/13 with conditions, and whether services to minors are offered at all.]

We do not knowingly collect personal information from children below the applicable age without appropriate consent/authorisation. If you believe a child has provided us with personal information contrary to this policy, please contact us and we will take appropriate steps.

⚠️ **COUNSEL** — confirm the age policy and any provision of services to minors, which raises additional requirements (UK Age Appropriate Design Code; US COPPA; parental/guardian consent; and clinical considerations). If minors are ever supported, this section and the underlying processes must be substantially expanded.

12. Automated decision-making

Faresay does **not** currently make decisions producing legal or similarly significant effects about you based **solely** on automated processing without human involvement. We may use limited automated processing (e.g. to suggest clinician matches), but clinical decisions are made by your clinician.

⚠️ **COUNSEL** — confirm this remains accurate as matching/recommendation features evolve; if any solely-automated significant decisions are introduced, additional UK GDPR Article 22 safeguards and disclosures will be required.

13. Data breaches

We maintain procedures to detect, investigate and respond to personal-data breaches. Where a breach is likely to result in a risk to your rights and freedoms, we will notify the relevant supervisory authority and affected individuals as required by law.

- **UK:** notification to the **ICO** without undue delay and, where required, within 72 hours of becoming aware, and to affected individuals where the breach is high-risk.
- **US** ⚠️ **COUNSEL:** where HIPAA applies, breach notification obligations under the **HIPAA Breach Notification Rule** (to individuals, HHS, and in some cases the media) and applicable **state breach-notification laws** must be followed.

Our incident-response approach is detailed in the [Security & Data Protection Policy](#).

14. How to complain ⚠️ COUNSEL

If you have a concern about how we handle your personal data, please contact us first (Section 17) so we can try to resolve it.

- **UK:** you have the right to lodge a complaint with the **Information Commissioner's Office (ICO)** — <https://ico.org.uk> — though we ask that you contact us first.
- **US:** depending on the issue and jurisdiction, you may be able to complain to relevant regulators, which may include the **US Department of Health and Human Services Office for Civil Rights (HHS OCR)** for HIPAA matters, your **State Attorney General**, and other applicable state regulators. [PLACEHOLDER: confirm relevant US regulators / contact routes.]

⚠️ **COUNSEL** — finalise the list of competent US regulators and complaint routes (including EU/EEA lead supervisory authority analysis if EU residents are served).

15. HIPAA Notice of Privacy Practices ⚠️ **COUNSEL**

Where HIPAA applies to Faresay's US operations, a separate **HIPAA Notice of Privacy Practices (NPP)** may be legally required and will describe how protected health information (PHI) may be used and disclosed and the rights individuals have under HIPAA. This Privacy Policy does **not** replace any required NPP.

⚠️ **COUNSEL** — confirm whether Faresay is a HIPAA covered entity, business associate, or neither for each US data flow; whether an NPP is required and, if so, who must provide it (Faresay and/or the clinician); and ensure appropriate **Business Associate Agreements (BAAs)** are in place. Link the NPP here once finalised:
[PLACEHOLDER: link to HIPAA Notice of Privacy Practices].

16. State-specific disclosures

[PLACEHOLDER: state-specific privacy disclosures to be inserted as the US footprint and applicable state laws are confirmed — e.g. California CCPA/CPRA notice at collection and "Your Privacy Choices", Washington My Health My Data Act consumer-health-data disclosures, and other state-required notices.] ⚠️ **COUNSEL**

17. Contact & updates

Privacy / data-protection contact: [PLACEHOLDER: privacy contact name, email, postal address] **Data Protection Officer (if appointed):** None appointed — no statutory DPO required at current scale; data-protection contact: Joel Harvey, support@faresay.com **UK / EU representative (if required)** ⚠️ **COUNSEL:** [PLACEHOLDER: representative details] **US privacy contact** ⚠️ **COUNSEL:** [PLACEHOLDER: US contact / toll-free number where required]

We may update this Privacy Policy from time to time. We will post the updated version with a revised "Last updated" date and, where changes are material, take additional steps to notify you as required by law.

This is a DRAFT document prepared for legal/privacy counsel review. It is not legal advice and must be reviewed and finalised by qualified UK and US counsel before publication or use.

Terms of Service

⚠️ DRAFT v0.1 — for legal counsel review. NOT legal advice. Counsel must finalise before use. Last updated: 26 June 2026

Faresay — Terms of Service

These Terms of Service ("**Terms**") govern your access to and use of the Faresay marketplace platform, including our website, applications, and related services (together, the "**Platform**"). The Platform is operated by Faresay Ltd, a company registered in [PLACEHOLDER: jurisdiction of incorporation] with registered office at 167-169 Great Portland Street, London W1W 5PF ("**Faresay**", "**we**", "**us**", "**our**").

Please read these Terms carefully. They apply to you whether you are located in the United Kingdom or, where and when the Platform is made available, in the United States.

⚠️ COUNSEL — Confirm the contracting entity for each market (UK operating entity now; US commercial entity / MSO, and the relationship to any "friendly PC", on US launch — see `business-model-canvas.md` and the friendly-PC structure in `CONTEXT.md`). These Terms may need a market-specific or jurisdiction-specific addendum or version.

1. Acceptance of these Terms

1.1 By creating an account, accessing, or using the Platform, you confirm that you have read, understood, and agree to be bound by these Terms and by any documents expressly incorporated by reference (including our Privacy Policy — see `privacy-policy.md`). If you do not agree, you must not use the Platform.

1.2 If you are using the Platform on behalf of another person, you represent that you are authorised to accept these Terms on their behalf. ⚠️ COUNSEL — confirm whether use on behalf of another adult is permitted at all, and how it interacts with consent and confidentiality.

1.3 We may require you to affirmatively accept these Terms (for example, by clicking "I agree") before you can use certain features. Your continued use of the Platform constitutes ongoing acceptance.

2. Description of the Service

2.1 **Faresay is a marketplace.** The Platform connects clients ("**you**", "**Client**") with independent, licensed mental-health professionals ("**Therapists**") and provides technology and supporting services that facilitate discovery, scheduling, communication, and payment.

2.2 **Faresay does NOT itself provide clinical, therapy, counselling, medical, or healthcare services.** Faresay is **not** a healthcare provider, a medical practice, or a provider of mental-health treatment. Faresay does not practise psychology, psychotherapy, counselling, social work, or medicine, and does not supervise, direct, or control the clinical judgement of any Therapist.

2.3 Therapists are **independent** practitioners. They are not employees or agents of Faresay. The clinical relationship — including all assessment, diagnosis, treatment, advice, and the clinical record — is **between you and your Therapist**, not between you and Faresay. See Section 7.

2.4 The services Faresay provides are **technology and marketing platform services**: software, a searchable directory and matching tools, scheduling, secure messaging, video-session facilitation, payment processing, and client-acquisition and support functions. ⚠️ COUNSEL — this characterisation is material to corporate-practice-of-medicine / corporate-practice-of-psychology and fee-splitting analysis (see `CONTEXT.md`); confirm wording with US healthcare-regulatory counsel.

2.5 We do not guarantee that any particular Therapist will be available, suitable, or a good match for you, that any course of therapy will achieve any particular outcome, or that you will be matched at all.

3. NOT AN EMERGENCY OR CRISIS SERVICE

⚠️ THE PLATFORM IS NOT FOR EMERGENCIES OR CRISES.

Faresay and the Platform are **NOT** an emergency service, crisis line, or suicide-prevention service. Therapists on the Platform are not available for emergency, urgent, or crisis response, and messages or bookings made through the Platform are **not** monitored for emergencies.

If you are experiencing a medical or mental-health emergency, are in crisis, or are at risk of harming yourself or others, do NOT use the Platform. Get help immediately:

United States - Call or text **988** (Suicide & Crisis Lifeline) - Call **911** for any life-threatening emergency

United Kingdom - Call **999** for any life-threatening emergency - Call **111** for urgent (non-life-threatening) NHS help - Call the **Samaritans** free on **116 123** (24 hours a day)

If you are outside these countries, contact your local emergency services.

⚠️ COUNSEL / ⚠️ CLINICAL — Verify all numbers and service descriptions before publication, and confirm they match the `crisis-safeguarding-policy.md`. Confirm 988 (US), 111 (UK NHS), and Samaritans 116 123 details and any region-specific equivalents.

3.1 You acknowledge that the Platform is intended for non-urgent mental-health support delivered by Therapists on a scheduled basis, and that response times for messages and bookings are not immediate.

4. Eligibility and Age

4.1 The Platform is intended for **adults**. You must be at least 18 years old (or the age of majority in your jurisdiction, if higher) to create an account and use the Platform.

4.2 **Minors.** ⚠️ COUNSEL / ⚠️ CLINICAL — Confirm whether services to minors will be offered at all and, if so, the consent model (parental/guardian consent, age thresholds for independent consent which vary by US state and within the UK, e.g. Gillette/Fraser considerations), verification, and safeguarding requirements. Until finalised, this draft assumes **adults only** and minors are out of scope.

4.3 By using the Platform, you represent and warrant that you meet the eligibility requirements and that the information you provide is accurate.

4.4 **Location.** Therapists must be appropriately licensed in the jurisdiction where you are located at the time of a session. You agree to provide accurate location information and to inform your Therapist if your location changes, because this may affect whether they can lawfully provide services to you. ⚠️ COUNSEL — ties to US per-state licensure and interstate compacts (PSYPACT, Counseling Compact, Social Work Licensure Compact) per `CONTEXT.md`.

5. Accounts and Registration

5.1 To use most features you must create an account. You agree to provide accurate, current, and complete information and to keep it up to date.

5.2 You are responsible for safeguarding your login credentials and for all activity that occurs under your account. Notify us promptly at [PLACEHOLDER: security/support contact] if you suspect any unauthorised use.

5.3 You may not share your account, create an account using false information, or create an account on behalf of another person without authorisation.

5.4 We may refuse, suspend, or terminate accounts in accordance with Section 17.

6. The Therapist Relationship

6.1 Therapists are **independent licensed professionals** who use the Platform to offer their services. They are solely responsible for the mental-health and clinical services they provide, including their professional judgement, the suitability and quality of care, their compliance with applicable licensing, professional, and ethical standards, and the maintenance of their clinical records.

6.2 **The clinical relationship is between you and your Therapist.** Faresay is not a party to that relationship. Faresay's role is limited to facilitating discovery, matching, scheduling, communication, and payment, and providing the supporting technology.

6.3 Faresay does not control or guarantee the qualifications, licensure status, conduct, advice, or performance of any Therapist beyond any onboarding and verification steps we describe. ⚠️ COUNSEL / ⚠️ CLINICAL — confirm what verification and credentialing Faresay does perform and represent (see `clinical-governance-policy.md` and `therapist-agreement.md`); align the representations here with what is actually done, to avoid creating liability.

6.4 Any agreement for clinical services, including any therapist-specific terms, informed-consent forms, or notices, is between you and the Therapist. Where there is a conflict between these Terms and a Therapist's clinical engagement documents on a clinical matter, the Therapist's clinical documents govern that clinical matter.

7. Informed Consent to Telehealth

7.1 The Platform supports the delivery of services by remote/telehealth means (for example, video, voice, or messaging). Telehealth has benefits and limitations and may not be appropriate for every person or condition.

7.2 Before or at the start of services, your Therapist is responsible for obtaining your **informed consent to telehealth** as required by applicable law and professional standards, including explaining the nature, benefits, risks, and limitations of remote care, alternatives, confidentiality, and what to do in an urgent situation.

7.3 By using the Platform to receive remote services, you acknowledge that care may be delivered remotely and that you may be asked to provide separate informed consent to your Therapist.

⚠️ COUNSEL / ⚠️ CLINICAL — Telehealth informed-consent requirements vary significantly by US state and differ in the UK. Confirm whether consent is captured by the Therapist, by the Platform, or both; whether a Platform-level telehealth consent is required; and the exact required disclosures per jurisdiction. Do not finalise this section without counsel and a clinical advisor.

8. No Professional Relationship with Faresay; Medical Disclaimer

8.1 No clinician–client relationship is formed with Faresay. Nothing on the Platform, and no interaction with Faresay or its staff, creates a clinician–client, doctor–patient, therapeutic, or other professional relationship between you and Faresay.

8.2 Faresay does not provide medical or clinical advice. Content made available by Faresay (such as general information, directory listings, or self-help material) is for general informational purposes only and is **not** a substitute for professional diagnosis, advice, or treatment. Always seek the advice of a qualified professional regarding any mental- or physical-health condition. Never disregard or delay seeking professional advice because of something accessed through the Platform.

8.3 Faresay does not recommend or endorse any specific Therapist, treatment, opinion, or course of action. Reliance on any information made available through the Platform is at your own risk.

9. Pricing and Payments

9.1 Session fees. Therapists set or agree the fees for their sessions, which are displayed to you before you book. The fee model is **cash-pay / out-of-network**: you pay out of pocket for sessions. ⚠️ COUNSEL — confirm fee-setting mechanics (Therapist-set vs Platform-set vs banded) and how this interacts with fee-splitting/CPOM analysis (see `CONTEXT.md`).

9.2 Insurance. Faresay does not bill insurance in the initial model. You are responsible for the full session fee. If you wish to seek reimbursement from an insurer, that is a matter between you and your insurer.

9.3 Platform fee. Faresay's marketplace take rate is **15%**, which is built into the model as a **technology and marketing platform fee** for the software, discovery, scheduling, payment, and client-acquisition services we provide. The 15% is not a split of the Therapist's professional fee. ⚠️ COUNSEL — confirm exactly how the 15% is presented to Clients (e.g., embedded in the displayed price vs shown as a separate line) and that the presentation is consistent with the fee characterisation in `CONTEXT.md` and applicable fee-splitting law.

9.4 Payment processing. Payments are processed through a third-party payment provider. By providing payment details, you authorise us (or our payment provider) to charge the applicable fees. You agree to the payment provider's terms. ⚠️ COUNSEL — identify provider and confirm flow of funds, especially under any MSO/friendly-PC structure.

9.5 Taxes. Prices are exclusive of any applicable taxes unless stated otherwise. You are responsible for any taxes that apply to you. ⚠️ COUNSEL — confirm VAT (UK) and US sales/tax treatment.

9.6 Cancellations and rescheduling. You may cancel or reschedule a booked session up to [PLACEHOLDER: cancellation window, e.g. 24 hours] before the session start time without charge. Cancellations or reschedules made within the cancellation window may be subject to a charge of up to [PLACEHOLDER: cancellation fee / full session fee], as disclosed at booking. ⚠️ COUNSEL — confirm the policy and that it is fairly disclosed (UK consumer-protection considerations below).

9.7 No-shows. If you fail to attend a booked session without cancelling within the permitted window, you may be charged up to the full session fee. ⚠️ COUNSEL — confirm.

9.8 Refunds. Refunds are handled as follows: [PLACEHOLDER: refund policy]. Where a session is not delivered due to the Therapist's or the Platform's fault, you will not be charged or will be refunded the session fee. ⚠️ COUNSEL — confirm refund policy and reconcile with UK consumer cancellation rights (e.g., Consumer Rights Act 2015 and Consumer Contracts Regulations 2013, including how the right to cancel interacts with services that begin within the cooling-off period) and US state requirements.

10. Client Responsibilities and Acceptable Use

10.1 You agree to: - (a) provide accurate information, including accurate location and any information reasonably needed for safe care; - (b) treat Therapists and Faresay staff with respect; - (c) attend booked sessions or cancel in accordance with Section 9; and - (d) use the Platform only for lawful purposes and in accordance with these Terms.

10.2 You agree **not** to: - (a) use the Platform for any emergency or crisis (see Section 3); - (b) impersonate any person or misrepresent your identity, age, or location; - (c) record, copy, or distribute sessions or communications without consent and lawful basis; - (d) harass, abuse, threaten, or harm any person; - (e) attempt to gain unauthorised access to the Platform, interfere with its operation, introduce malicious code, or scrape or harvest data; - (f) circumvent the Platform to arrange or pay for services off-platform in order to avoid fees, where prohibited; ⚠️ COUNSEL — confirm enforceability and wording of any anti-circumvention provision; - (g) use the Platform to infringe the rights of others or to post unlawful, defamatory, or infringing content; or - (h) use the Platform in violation of any applicable law or regulation.

10.3 We may remove content and suspend or terminate access for breach of this Section.

11. Intellectual Property

11.1 The Platform and all software, text, graphics, logos, trademarks, and other content provided by Faresay (excluding Therapist content and your content) are owned by or licensed to Faresay and protected by intellectual-property laws. "Faresay" and our logos are our trademarks.

11.2 We grant you a limited, non-exclusive, non-transferable, revocable licence to access and use the Platform for your personal, non-commercial use in accordance with these Terms. No other rights are granted.

11.3 **Your content.** You retain ownership of content you submit. You grant Faresay a non-exclusive, worldwide, royalty-free licence to host, store, and use your content solely as necessary to operate and provide the Platform and as described in our Privacy Policy. ⚠️ COUNSEL — ensure this licence does not extend to clinical-record content in a way that conflicts with the Therapist owning the clinical record (see `CONTEXT.md`) or with health-privacy law.

11.4 You must not use Faresay's intellectual property without our prior written consent.

12. Third-Party Providers and Links

12.1 The Platform relies on and may link to third-party services (for example, payment processors, video providers, and other vendors). Therapists are themselves independent third-party providers (see Sections 2 and 6).

12.2 We are not responsible for the acts, omissions, content, products, or services of any third party, including Therapists, and your dealings with them are at your own risk and subject to their terms. Inclusion of a link or provider is not an endorsement.

13. Disclaimers of Warranties

13.1 To the fullest extent permitted by law, the Platform is provided "**as is**" and "**as available**", without warranties of any kind, whether express, implied, or statutory, including implied warranties of merchantability, fitness for a particular purpose, and non-infringement.

13.2 We do not warrant that the Platform will be uninterrupted, error-free, secure, or free of harmful components, or that any defect will be corrected.

13.3 Faresay makes no warranty regarding clinical services. Because Faresay does not provide clinical services, we make no representation or warranty as to the quality, suitability, safety, or outcome of any service provided by any Therapist.

13.4 Statutory rights. ⚠️ COUNSEL — In the UK and certain US jurisdictions, some warranties and consumer rights cannot lawfully be excluded. This Section must be qualified so it does not purport to exclude rights that cannot be excluded (e.g., under the UK Consumer Rights Act 2015). Counsel to add appropriate "nothing in these Terms affects your statutory rights" wording and jurisdiction carve-outs.

14. Limitation of Liability

14.1 To the fullest extent permitted by law, Faresay and its officers, directors, employees, and agents will not be liable for any indirect, incidental, special, consequential, exemplary, or punitive damages, or for any loss of profits, revenue, data, goodwill, or other intangible losses, arising out of or relating to your use of (or inability to use) the Platform.

14.2 To the fullest extent permitted by law, Faresay's total aggregate liability arising out of or relating to these Terms or the Platform will not exceed the greater of (a) the total platform fees you paid to Faresay in the [PLACEHOLDER: e.g. 12] months before the event giving rise to the claim, or (b) [PLACEHOLDER: monetary cap].

⚠️ COUNSEL — set the cap and confirm enforceability per jurisdiction.

14.3 Clinical services. Faresay is not liable for the clinical services provided by Therapists, who are independent and solely responsible for their services (Section 6).

14.4 Carve-outs. ⚠️ COUNSEL — Nothing in these Terms limits or excludes liability that cannot lawfully be limited or excluded, including (UK) liability for death or personal injury caused by negligence, fraud or fraudulent misrepresentation, and any other liability that cannot be excluded under applicable law. Counsel to confirm the precise carve-outs for the UK and each US state.

15. Indemnification

15.1 To the fullest extent permitted by law, you agree to indemnify and hold harmless Faresay and its officers, directors, employees, and agents from and against any claims, liabilities, damages, losses, and expenses (including reasonable legal fees) arising out of or related to (a) your breach of these Terms, (b) your misuse of the Platform, or (c) your violation of any law or the rights of any third party. ⚠️ COUNSEL — confirm scope and enforceability against consumers (some jurisdictions limit consumer indemnities).

16. Dispute Resolution; Arbitration and Class-Action Waiver; Governing Law

16.1 Informal resolution. Before bringing a formal claim, you agree to contact us at [PLACEHOLDER: disputes contact] and attempt to resolve the dispute informally for at least [PLACEHOLDER: e.g. 30] days.

16.2 Governing law and venue. These Terms and any dispute arising out of them are governed by the laws of [PLACEHOLDER: governing law], and the courts of [PLACEHOLDER: venue] will have jurisdiction, except where mandatory local consumer law provides otherwise. ⚠️ COUNSEL — set governing law and venue separately for the UK and the US; confirm that consumers retain the protection of mandatory laws of their home jurisdiction.

16.3 Arbitration (US). ⚠️ COUNSEL — For US users, counsel may include a binding arbitration agreement (e.g., under the Federal Arbitration Act), specifying the arbitration provider and rules, seat/venue at [PLACEHOLDER: arbitration venue], a delegation clause, costs, and an opt-out mechanism. The enforceability and required form of

any arbitration clause is highly jurisdiction-specific and must be drafted by counsel. **This draft does not contain an operative arbitration agreement.**

16.4 Class-action waiver (US). ⚠️ COUNSEL — Any waiver of class, collective, or representative actions, and any jury-trial waiver, must be drafted and validated by US counsel for enforceability and required prominence/formatting. **This draft does not contain an operative class-action waiver.**

16.5 UK consumers. ⚠️ COUNSEL — Mandatory arbitration and class-action waivers are generally **not** appropriate or enforceable against UK consumers. UK consumers should retain the right to bring proceedings in their local courts. Counsel to ensure UK terms do not include unfair or unenforceable dispute clauses (Consumer Rights Act 2015 fairness rules).

17. Suspension and Termination

17.1 You may stop using the Platform and close your account at any time, subject to settling any amounts due and to your Therapist's separate clinical/discharge arrangements.

17.2 We may suspend or terminate your access, with or without notice, if (a) you breach these Terms, (b) we are required to do so by law, (c) we reasonably believe your use poses a risk to others or to the Platform, or (d) we discontinue the Platform or a feature.

17.3 On termination, the licences granted to you end. Sections that by their nature should survive (including Sections 8, 11, 13, 14, 15, 16, and 18) survive termination.

17.4 ⚠️ COUNSEL / ⚠️ CLINICAL — Termination must not abruptly cut off care in a clinically unsafe way. Coordinate with `clinical-governance-policy.md` and the Therapist's continuity-of-care obligations so that clients are signposted appropriately.

18. Changes to these Terms

18.1 We may update these Terms from time to time. If we make material changes, we will provide reasonable notice (for example, by email or an in-Platform notice) before they take effect.

18.2 Changes take effect on the date stated in the updated Terms. Your continued use of the Platform after the effective date constitutes acceptance. If you do not agree, you must stop using the Platform. ⚠️ COUNSEL — confirm change-of-terms mechanics meet consumer-protection requirements in each market.

19. Miscellaneous

19.1 **Entire agreement.** These Terms, together with documents incorporated by reference, are the entire agreement between you and Faresay regarding the Platform and supersede prior agreements on that subject.

19.2 **Severability.** If any provision is held unenforceable, the remaining provisions remain in effect, and the unenforceable provision will be modified to the minimum extent necessary.

19.3 **No waiver.** Our failure to enforce any provision is not a waiver of it.

19.4 **Assignment.** You may not assign these Terms without our consent. We may assign these Terms in connection with a merger, acquisition, or sale of assets, subject to applicable law.

19.5 **No agency.** Nothing in these Terms creates any partnership, joint venture, agency, or employment relationship between you and Faresay, or between Faresay and any Therapist.

19.6 **Force majeure.** We are not liable for any failure or delay caused by events beyond our reasonable control.

19.7 **Notices.** We may provide notices to you by email or through the Platform. You may contact us as set out in Section 20.

19.8 **Language.** ⚠️ COUNSEL — confirm whether localised/translated versions are needed and which language governs.

20. Contact

If you have questions about these Terms, contact us at:

- **Entity:** Faresay Ltd
- **Address:** 167-169 Great Portland Street, London W1W 5PF
- **Email:** [PLACEHOLDER: contact email, e.g. enquiries@ / legal@]

For urgent or emergency situations, do **not** contact us — see Section 3.

End of draft. ⚠️ This document is a first draft for legal counsel review and is not legal advice. Counsel must finalise before use.

Therapist Agreement

⚠️ DRAFT v0.1 — for legal counsel review. NOT legal advice. Counsel must finalise before use. Last updated: 26 June 2026

Faresay Therapist Agreement

This Therapist Agreement (the "**Agreement**") is entered into between Faresay and the independent mental-health professional who accepts it (the "**Therapist**"). It governs the Therapist's participation in the Faresay marketplace platform.

By clicking to accept, signing, or by providing services through the Faresay platform, the Therapist agrees to be bound by this Agreement.

1. Parties and Definitions

1.1 Parties

1.1.1 "**Faresay**" means Faresay Ltd, a private limited company registered in England and Wales under company number 17302034 whose registered office is at 167-169 Great Portland Street, London W1W 5PF (also referred to as "**we**", "**us**", "**our**", and the "**Platform Operator**").

⚠️ COUNSEL: For the US market this party may be the management services organisation ("MSO") entity rather than the UK operating company, and a separate professional entity ("friendly PC" / professional corporation) may be the contracting or supervising entity in corporate-practice states. Confirm the correct contracting party per jurisdiction before execution. See clause 2.6.

1.1.2 "**Therapist**" means the individual licensed mental-health professional identified in the onboarding registration, who is contracting in their own capacity (or, where permitted, through their own professional practice entity) (also referred to as "**you**" and "**your**").

1.1.3 Faresay and the Therapist are each a "**Party**" and together the "**Parties**".

1.2 Definitions

In this Agreement:

- "**Client**" means an individual who books, accesses, or receives clinical services from the Therapist via the Platform.
- "**Clinical Record**" means the clinical notes, assessments, treatment plans, and other professional records the Therapist creates in connection with the care of a Client.
- "**Clinical Services**" means the regulated mental-health / psychotherapy / counselling services the Therapist provides to Clients.
- "**Faresay Policies**" means the policies referenced in clause 18, including the Clinical Governance Policy (`clinical-governance-policy.md`), the Crisis/Safeguarding Policy (`crisis-safeguarding-policy.md`), the Privacy Policy (`privacy-policy.md`), the Security & Data Protection Policy (`security-data-protection-policy.md`), and the Terms of Service (`terms-of-service.md`), each as updated from time to time.

- **"Platform"** means the Faresay website, applications, software, and related technology, discovery, scheduling, payment, and marketing services.
- **"Platform Fee"** means the fee described in clause 4.2.
- **"Session Fee"** means the fee payable by a Client for a session of Clinical Services, as set in accordance with clause 4.1.
- **"Territory"** means the United Kingdom and, on expansion, the United States (the 50 states and the District of Columbia).

2. Nature of the Relationship

2.1 Independent contractor. The Therapist is an independent contractor and not an employee, worker, partner, joint venturer, or agent of Faresay. Nothing in this Agreement creates an employment or agency relationship. The Therapist is responsible for their own working methods, scheduling (subject to bookings accepted), and the manner in which Clinical Services are delivered, consistent with applicable professional standards.

2.2 The Therapist owns the clinical relationship. The professional relationship between the Therapist and each Client is exclusively the Therapist's. The Therapist exercises independent professional clinical judgement and is solely responsible for all clinical decisions. Faresay does not direct, control, or interfere with the Therapist's clinical judgement or the content of Clinical Services.

2.3 The Therapist owns the Clinical Record. The Therapist is the owner of (and, where applicable, the controller / custodian of) the Clinical Record, and is responsible for creating, maintaining, retaining, and securing it in accordance with applicable law and professional obligations. Faresay processes Client data as described in clause 9 and the Privacy Policy.

2.4 Faresay is a technology and marketing platform. Faresay provides software, client discovery and acquisition, scheduling, payment processing, and related administrative and marketing services. Faresay does not provide Clinical Services, does not employ the Therapist to provide Clinical Services, and is not a party to the clinical relationship.

2.5 Characterisation of the Platform Fee. The Platform Fee is a **fee for the technology and marketing platform and services Faresay provides** (software, discovery, scheduling, payments, client acquisition, and administrative support). It is **not** a division, split, or share of the Therapist's professional or clinical fee, and is not consideration for the referral of patients.

 **COUNSEL:** This characterisation is central to compliance with fee-splitting and anti-kickback rules and (in the US) corporate-practice-of-medicine / corporate-practice-of-psychology doctrines. The fee structure, the contracting entity, and this language must be reviewed and validated by US healthcare-regulatory counsel on a per-state basis, and by UK counsel, before use. Do not represent this characterisation as settled until confirmed.

2.6 US MSO / friendly-PC structure. In any US jurisdiction whose corporate-practice doctrine so requires, Clinical Services may be contracted through, or supervised by, a professional entity (a "friendly PC" / professional corporation), with Faresay (or its US affiliate) acting as the management services organisation providing non-clinical administrative, technology, and marketing services under a management services agreement. The allocation of clinical control to the professional entity and non-clinical control to the MSO is intended at all times to preserve the Therapist's clinical independence.

 **COUNSEL:** The MSO/friendly-PC structure, ownership, control, and the management-fee mechanics must be designed and validated by US healthcare-regulatory counsel for each state. See [clinical-governance-](#)

3. Eligibility and Credentialing

3.1 **Licensure.** The Therapist must hold, and maintain at all times, a valid, current, and unrestricted licence, registration, or accreditation to provide the relevant Clinical Services:

3.1.1 in the United Kingdom, with the applicable regulator or recognised professional body (for example HCPC, BACP, UKCP, BPS, or equivalent, as appropriate to the profession); and

3.1.2 in the United States, in the **state where the Client is located at the time of each session**, including by means of an interstate compact privilege where applicable (for example PSYPACT, the Counseling Compact, or the Social Work Licensure Compact).

⚠ COUNSEL: "Located where the Client is" is the prevailing telehealth licensure rule, but per-state nuances (temporary-practice rules, compact eligibility, and which professions each compact covers) must be confirmed by counsel. See `clinical-governance-policy.md` .

3.2 **Insurance.** The Therapist must hold and maintain professional indemnity / professional liability (malpractice) insurance meeting the requirements of clause 13 throughout the term.

3.3 **Background and identity checks.** The Therapist consents to, and must satisfactorily complete, identity verification, credential verification, and background checks as Faresay reasonably requires, including (in the UK) an enhanced DBS check where appropriate and (in the US) equivalent state background and exclusion-list screening.

3.4 **Ongoing monitoring.** The Therapist authorises Faresay to verify and to monitor on an ongoing basis the status of the Therapist's licences, registrations, insurance, and compact privileges, and to use third-party verification services for this purpose. The Therapist must notify Faresay **within [PLACEHOLDER: notice period, e.g. 3 business days]** of any change, lapse, suspension, condition, investigation, complaint, restriction, or revocation affecting any licence, registration, insurance, or fitness to practise.

3.5 **Accuracy.** The Therapist must ensure all credentialing information and profile content remains accurate, current, and not misleading.

4. Platform Services Provided to the Therapist

Subject to this Agreement, Faresay provides the Therapist with access to:

4.1 a professional profile and listing for client discovery and marketing; 4.2 scheduling, booking, and calendar tools; 4.3 secure messaging and a telehealth / video-session environment, or integration with one; 4.4 payment collection from Clients and payout processing to the Therapist; 4.5 administrative tooling to support record-keeping (noting the Therapist remains owner of the Clinical Record per clause 2.3); 4.6 customer-support and dispute-handling functions; and 4.7 such other features as Faresay may make available from time to time.

Faresay provides the Platform on an "as available" basis and may modify, add, or remove features, subject to clause 11.

5. Fees and Payment Flow

5.1 Session Fee. The Session Fee for each Client session is [PLACEHOLDER: set by the Therapist within Faresay parameters / set by Faresay / agreed]. In the initial US model, Clients pay out of pocket on a cash-pay / out-of-network basis; Faresay does not bill insurance.

5.2 Platform Fee (15%). Faresay retains a **Platform Fee of fifteen per cent (15%)** of the Session Fee for each session delivered through the Platform, as consideration for the technology and marketing services described in clause 4. As stated in clause 2.5, the Platform Fee is a platform/technology fee and **not** a split of the Therapist's professional fee.

5.3 Payout. Faresay (directly or through its payment processor) collects the Session Fee from the Client, deducts the Platform Fee, and pays the remaining eighty-five per cent (85%) to the Therapist, less any amounts properly withheld under this clause 5. Payouts are made [PLACEHOLDER: payout schedule / method] to the Therapist's nominated account.

5.4 Payment processor. Payments are processed by [PLACEHOLDER: payment processor, e.g. Stripe]. The Therapist must complete the processor's onboarding (including any required identity/KYC steps) and is subject to the processor's terms.

5.5 Refunds, cancellations, and chargebacks. Refunds and cancellations are handled in accordance with the Terms of Service and Faresay's cancellation policy. Where a Client refund, reversal, or chargeback relates to a session for which the Therapist has been (or is to be) paid, Faresay may deduct or recover the corresponding amount (including any associated processor fees) from current or future payouts. ⚠️ COUNSEL: confirm allocation of chargeback liability and processor fee pass-through.

5.6 Taxes. The Therapist is solely responsible for determining, reporting, and paying all taxes (including income tax, National Insurance / self-employment tax, VAT/sales tax where applicable) arising from the Therapist's income. Faresay does not withhold employment taxes. The Therapist must provide any tax information reasonably required (for example for [PLACEHOLDER: US 1099 / UK tax reporting] purposes).

5.7 No fee for unbooked time. Faresay pays only for Clinical Services actually delivered through the Platform. The Therapist is not paid for availability, profile time, or non-session activity.

6. Therapist Obligations

The Therapist must at all times:

6.1 Standard of care. Provide Clinical Services with the reasonable skill, care, and diligence expected of a competent professional in the relevant discipline, and in accordance with applicable professional, ethical, and regulatory standards.

6.2 Scope of practice and competence. Practise only within the scope of the Therapist's licence, training, and competence, and decline or refer where a Client's needs fall outside that scope (including where in-person or higher-acuity care is clinically indicated).

6.3 Telehealth compliance. Comply with all applicable telehealth / telemedicine laws, professional guidance, and technical-safety requirements in the Client's jurisdiction, including verifying the Client's location and identity at each session as required.

6.4 Informed consent. Obtain and document appropriate informed consent for assessment and treatment, including consent to telehealth delivery, before providing Clinical Services.

6.5 Confidentiality. Maintain the confidentiality of Client information in accordance with professional duties of confidence, this Agreement, and applicable data-protection law (clause 9), disclosing only as permitted or required by law or to protect against serious harm.

6.6 Mandatory reporting. Comply with all mandatory-reporting, duty-to-warn, and safeguarding obligations applicable in the relevant jurisdiction.

6.7 Crisis and safeguarding. Adhere to Faresay's Crisis/Safeguarding Policy ([crisis-safeguarding-policy.md](#)) and Clinical Governance Policy ([clinical-governance-policy.md](#)), each incorporated into this Agreement by reference, including escalation and risk-management procedures.

6.8 Record-keeping. Create and retain Clinical Records in accordance with applicable law and professional standards for the required retention period, and make them available to the Client (and, where lawfully required, to a continuing clinician) as required.

6.9 Conduct. Behave professionally toward Clients and Faresay staff, keep profile and marketing content accurate, and not make any false, misleading, or guaranteed-outcome claims.

6.10 Cooperation. Cooperate with Faresay's quality-assurance, clinical-governance, complaints, and audit processes.

7. Client Relationship and Non-Circumvention

7.1 Introductions. Faresay invests in client acquisition and marketing. Clients introduced to the Therapist through the Platform are introduced in consideration of the Therapist's participation in, and use of, the Platform.

7.2 Non-circumvention. During the term and for [PLACEHOLDER: period, e.g. 12 months] afterwards, the Therapist must not, for any Client first introduced to the Therapist through the Platform, solicit or arrange for Clinical Services to be provided and paid for outside the Platform with the primary purpose of avoiding the Platform Fee.

 **COUNSEL:** Non-circumvention / non-solicitation terms must respect (a) the Client's right to choose and continue with their clinician, (b) continuity-of-care and professional-ethics obligations, and (c) restraint-of-trade / enforceability limits under UK law and individual US state law. This clause must not be drafted or enforced so as to compromise patient care or abandon Clients. Counsel to confirm scope, duration, and carve-outs.

7.3 Continuity carve-out. Nothing in clause 7.2 prevents the Therapist from continuing to treat a Client where required by professional ethics or continuity-of-care obligations, or where the Client independently chooses to continue treatment; clause 7.2 addresses only deliberate fee-avoidance.

8. (Reserved)

This clause number is intentionally reserved.

9. Data Protection and Confidentiality

9.1 Roles. With respect to Client personal data, the Parties acknowledge that the Therapist (as the clinician owning the clinical relationship and Clinical Record) and Faresay (as the platform operator) each process personal data, and that their respective roles as controller, joint controller, or processor must be clearly mapped per processing activity. The detailed allocation is set out in the Privacy Policy ([privacy-policy.md](#)) and the Security & Data Protection Policy ([security-data-protection-policy.md](#)).

 **COUNSEL:** The controller / joint-controller / processor mapping is legally significant and fact-specific. Counsel and the DPO must confirm the mapping and put in place the correct UK GDPR Article 26 / Article 28 arrangements (and US equivalents) before launch.

9.2 **UK GDPR.** In the UK, both Parties must comply with the UK GDPR and the Data Protection Act 2018. Client clinical data is special-category (health) data requiring an appropriate lawful basis and Article 9 condition, and a high standard of security and confidentiality.

9.3 **HIPAA (US).** On US expansion, where Faresay acts as a business associate (or the Therapist/professional entity as a covered entity), the Parties must enter into a **HIPAA Business Associate Agreement** governing the use and protection of protected health information.

⚠ **COUNSEL:** Covered-entity vs business-associate status, and the form of the Business Associate Agreement, must be confirmed and drafted by US counsel. Also consider state health-privacy laws (e.g. Washington My Health My Data Act, CCPA/CPRA). See `privacy-policy.md`.

9.4 **Security.** The Therapist must comply with the Security & Data Protection Policy, use only Faresay-approved or otherwise compliant tools to handle Client data, and report any personal-data breach to Faresay without undue delay and in any event within [PLACEHOLDER: e.g. 24 hours] of becoming aware of it.

9.5 **Confidential information.** Each Party must keep confidential the other's non-public business information and use it only to perform this Agreement. This clause 9.5 does not limit the Therapist's separate professional duty of clinical confidentiality owed to Clients.

10. Intellectual Property and Licence

10.1 **Faresay IP.** Faresay (and its licensors) own all rights in the Platform, the Faresay name, logos, and marks, and all Faresay-created content. No rights are granted except as expressly set out here.

10.2 **Licence to the Therapist.** Faresay grants the Therapist a non-exclusive, non-transferable, revocable licence to use the Platform, and to use Faresay marks and materials, solely to provide and promote Clinical Services through the Platform and in accordance with Faresay's brand guidelines.

10.3 **Therapist content.** The Therapist retains ownership of content they supply (such as profile text and credentials) and grants Faresay a non-exclusive, worldwide, royalty-free licence to host, display, reproduce, and use that content to operate and market the Platform and the Therapist's listing.

10.4 **Clinical Record.** For the avoidance of doubt, this clause 10 does not affect the Therapist's ownership of the Clinical Record under clause 2.3.

11. Representations and Warranties

11.1 The Therapist represents and warrants that, on an ongoing basis:

11.1.1 they hold all licences, registrations, and compact privileges required to provide the Clinical Services in each relevant jurisdiction (clause 3); 11.1.2 they are not subject to any unspent disqualification, suspension, exclusion, debarment, or restriction that would prevent them from providing Clinical Services; 11.1.3 they hold the insurance required by clause 13; 11.1.4 all information they provide to Faresay is accurate and not misleading; and 11.1.5 they will comply with all applicable laws, professional standards, and Faresay Policies.

11.2 Except as expressly stated, the Platform is provided "as is" and Faresay disclaims all other warranties to the maximum extent permitted by law.

12. Indemnification

12.1 The Therapist will indemnify and hold harmless Faresay and its affiliates, and their officers and staff, from and against losses, liabilities, damages, claims, and reasonable costs (including legal fees) arising out of or in connection with:

12.1.1 the Therapist's provision of (or failure to provide) Clinical Services, including any clinical negligence, malpractice, or breach of professional duty; 12.1.2 the Therapist's breach of this Agreement, the Faresay Policies, or applicable law; 12.1.3 any claim that the Therapist lacked the required licensure, registration, or competence; and 12.1.4 the Therapist's breach of data-protection or confidentiality obligations.

12.2 Faresay will indemnify the Therapist for losses arising from Faresay's breach of this Agreement or its negligence in operating the Platform, subject to clause 13 (limitation of liability).

 **COUNSEL:** Mutual scope, caps, carve-outs, and the interaction with insurance and with the limitation of liability must be reviewed for each jurisdiction. Certain liabilities (e.g. clinical negligence to a Client) cannot be contracted away and must be backed by insurance.

13. Insurance Requirements

13.1 The Therapist must, at their own cost, hold and maintain professional indemnity / professional liability (malpractice) insurance with a reputable insurer, with cover of at least [PLACEHOLDER: minimum cover amount and basis, e.g. per-claim / aggregate] appropriate to the Clinical Services and each jurisdiction in which Clients are located.

13.2 The Therapist must provide evidence of cover on onboarding and on request, and must notify Faresay promptly if cover lapses, is reduced, or is cancelled.

13.3 Cover must remain in force during the term and, where the policy is claims-made, for [PLACEHOLDER: run-off / tail period] after termination.

14. Limitation of Liability

14.1 Nothing in this Agreement limits or excludes liability that cannot lawfully be limited or excluded, including liability for death or personal injury caused by negligence, for fraud, or any other liability that cannot be limited under applicable law.

14.2 Subject to clause 14.1, Faresay is not liable for indirect or consequential loss, loss of profit, loss of goodwill, or loss of anticipated savings.

14.3 Subject to clauses 14.1 and 14.2, Faresay's total aggregate liability arising out of or in connection with this Agreement is limited to [PLACEHOLDER: cap, e.g. the total Platform Fees paid by/attribution to the Therapist in the [12] months before the claim].

14.4 For the avoidance of doubt, Faresay is not responsible for the Clinical Services or for the Therapist's clinical decisions, which are the Therapist's sole responsibility.

 **COUNSEL:** Caps and exclusions must be reviewed for enforceability under UK law (e.g. Unfair Contract Terms Act 1977) and applicable US state law, and reconciled with the indemnity (clause 12) and insurance (clause 13).

15. Continuity of Care on Termination

15.1 The welfare of Clients takes priority on any termination or suspension. The Therapist must not abandon a Client and must comply with their professional continuity-of-care obligations.

15.2 On notice of termination (by either Party), the Parties will cooperate in good faith to manage an orderly transition, which may include:

15.2.1 the Therapist completing or safely concluding in-progress care, or arranging an appropriate handover or referral; 15.2.2 the Therapist providing the Client (and any continuing clinician the Client chooses) with access to the relevant Clinical Record as required by law and professional standards; and 15.2.3 Faresay supporting affected Clients in finding an alternative clinician on the Platform where the Client wishes.

15.3 The Therapist must give Clients reasonable notice of their departure where clinically appropriate, in a manner agreed with Faresay so as not to breach clause 7 while fully respecting Client choice and clause 15.1.

⚠ COUNSEL / ⚠ CLINICAL: Continuity-of-care, record-handover, and Client-notification mechanics must align with `clinical-governance-policy.md`, `crisis-safeguarding-policy.md`, and professional ethics rules in each jurisdiction.

16. Term, Suspension, and Termination

16.1 **Term.** This Agreement starts when the Therapist accepts it and continues until terminated under this clause 16.

16.2 **Termination for convenience.** Either Party may terminate on [PLACEHOLDER: notice period, e.g. 30 days'] written notice, subject to clause 15 (continuity of care).

16.3 **Termination for cause.** Faresay may suspend access or terminate immediately on written notice if the Therapist:

16.3.1 loses, or has restricted, any required licence, registration, compact privilege, or insurance; 16.3.2 is subject to a regulatory, safeguarding, or fitness-to-practise concern that Faresay reasonably considers presents a risk to Clients; 16.3.3 commits a material breach of this Agreement or the Faresay Policies (and, if remediable, fails to remedy it within [PLACEHOLDER: cure period]); or 16.3.4 becomes insolvent or ceases to practise.

16.4 **Suspension.** Faresay may suspend the Therapist's access pending investigation of any matter under clause 16.3 where reasonably necessary to protect Clients, acting in good faith.

16.5 **Effect of termination.** On termination, the licence in clause 10.2 ends and the Therapist must stop using Faresay marks. Accrued payment rights, and clauses intended to survive (including clauses 2.3, 5.5–5.6, 7, 9, 11–15, 17, 19, and 20), survive termination.

17. Compliance with Faresay Policies

17.1 The Therapist must comply with all Faresay Policies, which are incorporated into this Agreement by reference, including:

- Clinical Governance Policy — `clinical-governance-policy.md`
- Crisis/Safeguarding Policy — `crisis-safeguarding-policy.md`
- Privacy Policy — `privacy-policy.md`
- Security & Data Protection Policy — `security-data-protection-policy.md`
- Terms of Service — `terms-of-service.md`

17.2 Faresay may update the Faresay Policies from time to time in line with clause 20.2. Where a Policy conflicts with this Agreement on a clinical-safety or legal-compliance matter, the more protective requirement applies; otherwise this Agreement prevails.

18. Dispute Resolution and Governing Law

18.1 The Parties will first attempt in good faith to resolve any dispute through discussion and, if needed, [PLACEHOLDER: escalation / mediation process].

18.2 This Agreement and any dispute arising out of it are governed by the laws of [PLACEHOLDER: governing law, e.g. England and Wales for UK Therapists; relevant US state law for US Therapists], and the Parties submit to the [PLACEHOLDER: exclusive / non-exclusive] jurisdiction of the courts of [PLACEHOLDER: venue].

⚠️ COUNSEL: Governing law, venue, and any arbitration / class-action-waiver provisions differ materially between the UK and individual US states and must be set by counsel. Consider separate jurisdiction-specific schedules (UK vs US) rather than a single governing-law clause.

19. Notices

19.1 Notices under this Agreement must be in writing and sent to: (a) Faresay at [PLACEHOLDER: legal/notices email and address]; and (b) the Therapist at the contact details in their account.

19.2 Routine operational communications may be given through the Platform or by email and are deemed received [PLACEHOLDER: e.g. on the next business day].

20. Entire Agreement, Amendments, and General

20.1 **Entire agreement.** This Agreement and the documents it incorporates by reference are the entire agreement between the Parties on its subject matter and supersede prior discussions.

20.2 **Amendments.** Faresay may amend this Agreement or the Faresay Policies on [PLACEHOLDER: notice period] notice; continued use of the Platform after the effective date constitutes acceptance. Material changes adverse to the Therapist will be notified, and the Therapist may terminate under clause 16.2 if they do not accept them. ⚠️ COUNSEL: confirm unilateral-amendment mechanics are enforceable in each jurisdiction.

20.3 **Assignment.** The Therapist may not assign this Agreement without Faresay's consent. Faresay may assign to an affiliate or successor (including the US MSO/PC entities under clause 2.6).

20.4 **No partnership/agency.** Nothing creates a partnership or agency (clause 2.1).

20.5 **Severance.** If any provision is unenforceable, the rest remains in effect and the provision is modified to the minimum extent necessary.

20.6 **Waiver.** A failure to enforce any right is not a waiver of it.

20.7 **Third-party rights.** Except as expressly stated, no third party has rights to enforce this Agreement [PLACEHOLDER: confirm Contracts (Rights of Third Parties) Act 1999 position for UK].

End of draft. ⚠️ This document is a v0.1 first draft and must be reviewed and finalised by qualified UK and US legal counsel before use. It is not legal advice.

Data Processing Agreement (UK GDPR Art 28)

⚠️ DRAFT v0.1 — template for professional sign-off. NOT legal advice. A solicitor must finalise before reliance. Last updated: 25 June 2026

Faresay Data Processing Agreement

This Data Processing Agreement ("**DPA**") forms part of, and is incorporated into, the [Faresay Therapist Agreement](#) and the [Terms of Service](#) (together, the "**Agreement**") between Faresay and the therapist, practitioner or practice that subscribes to the Faresay platform (the "**Customer**").

It governs the processing of personal data carried out by Faresay on the Customer's behalf and is designed to satisfy **Article 28 of the UK GDPR** and the **Data Protection Act 2018**.

Roles. Under this DPA the **Customer is the Controller** of the clinical and client personal data they manage on the platform, and **Faresay is the Processor**, acting only on the Customer's documented instructions. Faresay is an independent Controller only for the limited account-administration, billing and security data described in Annex 1. This split reflects Faresay's product model: the therapist owns the client relationship and the clinical record; Faresay provides the software.

1. Definitions

Terms not defined here have the meaning given in the UK GDPR. In this DPA:

- "**UK GDPR**" means Regulation (EU) 2016/679 as it forms part of UK law by virtue of the European Union (Withdrawal) Act 2018, together with the Data Protection Act 2018.
- "**Controller**", "**Processor**", "**Data Subject**", "**Personal Data**", "**Special Category Data**", "**Processing**", "**Personal Data Breach**" have the meanings in the UK GDPR.
- "**Customer Personal Data**" means Personal Data that Faresay Processes on the Customer's behalf under the Agreement, as described in **Annex 1**.
- "**Sub-processor**" means any third party engaged by Faresay to Process Customer Personal Data. The current list is at **Annex 3** ([Sub-processor List](#)).
- "**TOMs**" means the technical and organisational measures at **Annex 2** ([Technical & Organisational Measures](#)).

2. Subject matter, duration, nature and purpose

Item	Detail
Subject matter	Provision of the Faresay practice-management platform to the Customer.
Duration	For the term of the Agreement, plus any post-termination period in Section 11.
Nature	Hosting, storage, transmission, scheduling, payment facilitation, video session hosting, messaging, reminders and related processing operations.

Item	Detail
Purpose	To enable the Customer to run their therapy practice (manage clients, book and deliver sessions, take payment, send reminders).
Categories of Data Subject	The Customer's clients; the Customer's own staff/associates where applicable.
Categories of Personal Data	Identity and contact details; appointment and scheduling data; payment metadata; messages; client-supplied intake responses; special-category health data (mental-health information).

The full detail is set out in **Annex 1**.

3. Processing only on documented instructions

3.1 Faresay shall Process Customer Personal Data **only on the documented instructions of the Customer**, including as set out in the Agreement, this DPA, and as given through the platform's features, unless required to do otherwise by UK law (in which case Faresay shall, where legally permitted, inform the Customer first).

3.2 Faresay shall **immediately inform** the Customer if, in its opinion, an instruction infringes the UK GDPR or other data-protection law.

3.3 Faresay shall not sell Customer Personal Data, nor use it for its own purposes, profiling, marketing, or to train AI models.

4. Confidentiality

Faresay shall ensure that persons authorised to Process Customer Personal Data are bound by an appropriate duty of confidentiality and Process the data only as instructed. Access is granted on a least-privilege, need-to-know basis (see Annex 2).

5. Security (Article 32)

5.1 Faresay shall implement and maintain the technical and organisational measures in **Annex 2**, appropriate to the risk, taking into account the special-category nature of the data.

5.2 Faresay may update the TOMs from time to time provided the level of protection is not materially reduced.

6. Sub-processors

6.1 The Customer **grants general authorisation** for Faresay to engage the Sub-processors listed in **Annex 3**.

6.2 Faresay shall impose data-protection obligations on each Sub-processor that are no less protective than those in this DPA, by written contract, and remains **fully liable** to the Customer for each Sub-processor's performance.

6.3 Faresay shall give the Customer **at least 14 days' notice** (by email and/or in-product) before adding or replacing a Sub-processor. The Customer may object on reasonable data-protection grounds; if the parties cannot resolve the objection, the Customer may terminate the affected service.

7. Data-subject rights

Taking into account the nature of the Processing, Faresay shall assist the Customer by appropriate technical and organisational measures, insofar as possible, to fulfil the Customer's obligation to respond to requests to exercise Data Subject rights (access, rectification, erasure, restriction, portability, objection). Where a Data Subject contacts Faresay directly about Customer Personal Data, Faresay shall, without undue delay, refer them to the Customer.

8. Personal Data Breach

8.1 Faresay shall notify the Customer **without undue delay, and in any event within 48 hours**, after becoming aware of a Personal Data Breach affecting Customer Personal Data.

8.2 The notification shall describe, to the extent known: the nature of the breach; categories and approximate number of Data Subjects and records affected; likely consequences; and measures taken or proposed. Faresay shall provide reasonable assistance to the Customer in meeting the Customer's own breach-notification duties to the ICO and Data Subjects (UK GDPR Arts 33–34).

9. DPIAs and prior consultation

Faresay shall provide the Customer with reasonable assistance with data-protection impact assessments (Art 35) and prior consultation with the ICO (Art 36), taking into account the nature of Processing and the information available to Faresay. A platform-level DPIA is maintained at [Data Protection Impact Assessment](#).

10. International transfers

10.1 Faresay primarily stores Customer Personal Data on infrastructure located in the **UK / EEA** (see Annex 3).

10.2 Where any transfer of Customer Personal Data to a third country occurs (for example via a Sub-processor), Faresay shall ensure an appropriate safeguard is in place — the **UK International Data Transfer Agreement (IDTA)**, the **UK Addendum to the EU Standard Contractual Clauses**, or an adequacy regulation — together with any supplementary measures required following a transfer risk assessment.

11. Return and deletion

11.1 On termination of the Agreement, Faresay shall, at the Customer's choice, **return or delete** all Customer Personal Data, and delete existing copies, unless UK law requires storage.

11.2 The Customer may **export their data at any time** during the term via the platform's export features. Faresay shall make Customer Personal Data available for export for **30 days** after termination, after which it shall be deleted within a further **90 days**, subject to routine backup-rotation cycles.

12. Audit

12.1 Faresay shall make available to the Customer all information reasonably necessary to demonstrate compliance with Article 28 and this DPA.

12.2 Faresay shall allow for and contribute to audits, including inspections, conducted by the Customer or an auditor it mandates, on reasonable prior notice (at least 30 days), no more than once per 12-month period (save where required by a supervisory authority or following a breach), subject to confidentiality. Faresay may satisfy audit requests by providing its security documentation, the TOMs, and responses to a reasonable security questionnaire.

13. Liability and precedence

13.1 The liability provisions of the Agreement apply to this DPA.

13.2 In the event of conflict between this DPA and the rest of the Agreement on data-protection matters, **this DPA prevails.**

14. Governing law

This DPA is governed by the laws of **England and Wales** and subject to the exclusive jurisdiction of its courts. *England and Wales (solicitor to confirm any Scotland/NI variation if serving those nations)*

Annex 1 — Details of Processing

- **Controller:** the Customer (therapist / practice).
- **Processor:** Faresay Ltd, a company registered in England and Wales (company number 17302034), registered office 167-169 Great Portland Street, London W1W 5PF.
- **Subject matter, duration, nature, purpose:** as Section 2.
- **Categories of Data Subject:** the Customer's clients; the Customer's staff/associates where applicable.
- **Categories of Personal Data:** name, email, phone; date of birth (optional); appointment dates/times; session/booking history; payment metadata (card details are handled by Stripe — Faresay does not store card numbers); messages between Customer and client; client intake/consent responses; **special-category health data** (mental-health information the Customer or client records).
- **Special-category data:** Yes — health data. Processed because it is necessary for the provision of health/social-care services by a health professional under a duty of confidentiality (UK GDPR Art 9(2)(h)) and/or with explicit consent (Art 9(2)(a)), as determined by the Controller.
- **Faresay-as-Controller data (outside this DPA):** the Customer's own account/login credentials, subscription and billing records, platform audit logs, aggregate/cookieless usage analytics, and security telemetry — Processed by Faresay as an independent Controller under the [Privacy Policy](#).

Annex 2 — Technical & Organisational Measures

See [Technical & Organisational Measures](#).

Annex 3 — Sub-processors

See [Sub-processor List](#).

This DPA is a draft template prepared to support a UK launch. It must be reviewed and finalised by a qualified data-protection solicitor before it is offered to customers.

Technical & Organisational Measures (Annex 2)

⚠️ DRAFT v0.1 — template for professional sign-off. NOT legal advice. Some controls describe target state; each is flagged. Last updated: 25 June 2026

Faresay Technical & Organisational Measures (TOMs)

This document is **Annex 2** to the [Data Processing Agreement](#). It describes the technical and organisational security measures Faresay maintains under **Article 32 of the UK GDPR**, appropriate to the risk of processing **special-category (mental-health) data**.

Faresay is early-stage and bootstrapped. Controls marked **[target]** are planned or partially implemented and being matured; all others are in place today. This annex is reviewed at least annually and on material change.

1. Pseudonymisation and encryption

- **Encryption in transit:** all traffic to and from the platform is served over **TLS 1.2+ (HTTPS)**. Video sessions use the encrypted transport provided by the video sub-processor.
- **Encryption at rest:** the primary database (Neon Postgres) and object/file storage encrypt data at rest using the provider's managed encryption (AES-256).
- **Secrets:** API keys and credentials are held as environment secrets in the hosting platform, never committed to source control.
- **Card data:** never stored by Faresay — card details are tokenised by **Stripe** (PCI-DSS Level 1 provider).

2. Confidentiality

- **Access control:** access to production data is least-privilege and limited to personnel who require it. Authentication to the platform is handled by **Clerk** with support for strong passwords and multi-factor authentication.
- **Therapist–client isolation:** application-level authorisation ensures a Customer can access only their own clients, sessions and records.
- **Staff confidentiality:** all personnel with data access are bound by confidentiality obligations.
- **Background checks [target]:** appropriate vetting for any future staff with production access.

3. Integrity

- **Audit logging:** security-relevant events and administrative actions are logged.
- **Change control:** code changes go through version control (Git) and review before deploying to production; infrastructure is managed as code where practical.
- **Input validation:** server-side validation and rate-limiting on public endpoints (e.g. self-booking) to mitigate abuse and injection.

- **Dependency hygiene [target]:** automated dependency and vulnerability scanning.

4. Availability and resilience

- **Managed infrastructure:** hosting (Vercel) and database (Neon) are managed, redundant cloud services with provider-level high availability.
- **Backups:** the database provider maintains automated backups and point-in-time recovery.
- **Restore testing [target]:** periodic test of backup restoration.

5. Resilience and recovery (Art 32(1)(c))

- **Incident response:** a documented process to detect, triage, contain and notify on a Personal Data Breach, including the **48-hour** Customer-notification commitment in the DPA.
- **Business continuity [target]:** documented recovery objectives (RPO/RTO) as the service scales.

6. Testing and evaluation (Art 32(1)(d))

- **Review cadence:** TOMs and security posture reviewed at least annually.
- **Penetration testing [target]:** independent testing prior to, or shortly after, scaled launch.

7. Data minimisation and retention

- Only data necessary to run the practice is collected. Health data is recorded at the Customer's discretion.
- Retention and deletion follow the DPA (export window + deletion timeline) and the [Privacy Policy](#).

8. Sub-processor security

- Sub-processors are selected for their security posture and bound by contractual data-protection obligations no less protective than the DPA. See [Sub-processor List](#).

9. Organisational measures

- **Data protection by design and default** is applied to new features (e.g. cookieless analytics; SMS used only for appointment reminders).
- **Policies:** this annex is supported by the [Security & Data Protection Policy](#), [Clinical Governance Policy](#) and [Crisis & Safeguarding Policy](#).
- **ICO registration:** Faresay is registered (or registering) with the ICO and pays the data-protection fee. *ICO registration applied for 26 June 2026 — registration number to follow.*

Draft prepared to support a UK launch. To be validated by a security specialist and data-protection solicitor before reliance.

Sub-processor List (Annex 3)

⚠️ DRAFT v0.1 — template for professional sign-off. NOT legal advice. Provider regions/terms must be verified against each provider's current DPA. Last updated: 25 June 2026

Faresay Sub-processor List

This document is **Annex 3** to the [Data Processing Agreement](#). It lists the third parties Faresay engages to Process Customer Personal Data, the purpose, the data involved, the hosting location, and the international-transfer safeguard where data leaves the UK/EEA.

Customers receive **at least 14 days' notice** before a new sub-processor is added (DPA Section 6). To subscribe to change notices: email to the address on the Customer's account; changes are also posted in-product.

Verify before reliance: each provider's processing region and current data-processing terms must be confirmed at finalisation — providers change defaults and sub-processor terms over time. Where a US transfer applies, the safeguard is the **UK IDTA** or the **UK Addendum to the EU SCCs**, supported by a transfer risk assessment.

Infrastructure sub-processors

Sub-processor	Purpose	Data processed	Location	Transfer safeguard
Neon	Primary database hosting (Postgres)	All structured platform data	AWS London (eu-west-2) — UK/EEA <i>[verify]</i>	UK/EEA — none required
Vercel	Application hosting & serverless compute	Transient request data in processing	Global edge; US parent	UK Addendum / IDTA

Application sub-processors

Sub-processor	Purpose	Data processed	Location	Transfer safeguard
Clerk	Authentication & identity	Account credentials, name, email	US <i>[verify]</i>	UK Addendum / IDTA
Stripe	Payment processing & subscriptions	Payment metadata; card data (tokenised by Stripe — not stored by Faresay)	Stripe Payments Europe (Ireland) + US	SCCs / UK Addendum
Daily.co	Video session hosting	Live audio/video session media (not stored by Faresay); display names	US <i>[verify]</i>	UK Addendum / IDTA

Sub-processor	Purpose	Data processed	Location	Transfer safeguard
Resend	Transactional email (bookings, reminders, receipts)	Recipient email; message content (appointment details)	US <i>[verify]</i>	UK Addendum / IDTA
Twilio	SMS appointment reminders	Client mobile number; reminder text	US / Ireland <i>[verify]</i>	SCCs / UK Addendum

Analytics sub-processors

Sub-processor	Purpose	Data processed	Location	Transfer safeguard
Plausible Analytics	Cookieless web analytics	Aggregate usage only — no cookies, no personal data, no cross-site tracking	EU (EU-hosted) <i>[verify]</i>	UK/EEA — none required

Notes

- **Card details** are never stored by Faresay — they are entered directly into Stripe's PCI-DSS-compliant systems and returned to Faresay only as a token.
- **Health data** is held in the primary database (Neon) and transmitted through the application and email/SMS sub-processors only to the extent the Customer's chosen features require (e.g. an appointment reminder contains the practitioner name and time, not clinical content).
- **Plausible** is used specifically because it is cookieless and processes no personal data, minimising Faresay's analytics footprint.

Draft prepared to support a UK launch. Regions and transfer mechanisms to be confirmed by a data-protection solicitor before reliance.

Record of Processing Activities (Art 30)



DRAFT v0.1 — template for professional sign-off. NOT legal advice. To be maintained as a living record. Last updated: 25 June 2026

Faresay Record of Processing Activities (ROPA)

This is Faresay's record under **Article 30 of the UK GDPR**. Faresay holds two roles:

- **As Processor** (Art 30(2)) — for the client/clinical data it processes on behalf of Customers (therapists).
- **As Controller** (Art 30(1)) — for its own account-administration, billing, security and analytics data.

It is a living document, reviewed at least annually and on material change.

Controller / Processor identity

- **Organisation:** Faresay Ltd (company number 17302034).
- **Registered address:** 167-169 Great Portland Street, London W1W 5PF.
- **ICO registration number:** applied for 26 June 2026 — registration number to follow.
- **Data protection contact:** Joel Harvey, Founder — support@faresay.com.

Part A — Processing as PROCESSOR (on behalf of Customers)

Field	Detail
Controllers on whose behalf we act	Faresay's subscribing therapists / practices ("Customers").
Categories of processing	Hosting, storage, transmission, scheduling, payment facilitation, video hosting, messaging, email/SMS reminders, data export and deletion.
Categories of Data Subject	Customers' clients; Customers' staff/associates.
Categories of Personal Data	Identity & contact (name, email, phone); date of birth (optional); appointment/scheduling data; payment metadata; messages; intake/consent responses; special-category health data .
Special-category data	Health (mental-health) data — Art 9(2)(h)/(a) as determined by the Controller.
Transfers to third countries	Via sub-processors only; safeguarded by UK IDTA / UK Addendum to EU SCCs. See Sub-processor List .
Retention	For the term; export available 30 days post-termination; deletion within a further 90 days (subject to backup rotation).

Field	Detail
Security measures	See Technical & Organisational Measures .

Part B — Processing as CONTROLLER (Faresay's own purposes)

Purpose	Data subjects	Personal data	Lawful basis	Retention
Account administration	Customers (therapists)	Name, email, login credentials, practice details	Contract (Art 6(1)(b))	Life of account + 12 months
Subscription & billing	Customers	Subscription status, payment metadata, invoices	Contract; legal obligation (tax records)	6 years (UK tax)
Platform security & audit logs	Customers; clients	IP, device, timestamps, security events	Legitimate interests (Art 6(1)(f)) — security	12 months
Service emails (transactional)	Customers	Email, account events	Contract	Life of account
Product analytics	Website visitors	Aggregate, cookieless — no personal data (Plausible)	Legitimate interests	N/A (no personal data)
Marketing / newsletter <i>(if used)</i>	Subscribers	Email	Consent (Art 6(1)(a))	Until unsubscribe
Support & dispute handling	Customers; clients	Correspondence	Legitimate interests; legal claims	to be confirmed

Transfers summary

Primary storage is **UK/EEA** (Neon, London). Where a sub-processor processes data outside the UK/EEA, transfers are safeguarded by the **UK IDTA** or **UK Addendum to the EU SCCs**, supported by a transfer risk assessment. Full mapping: [Sub-processor List](#).

Lawful basis for special-category data

For client health data processed **as Processor**, the lawful basis is determined by the Customer (Controller) — typically Art 9(2)(h) (provision of health/social-care by a professional under a confidentiality duty) and/or Art 9(2)(a) (explicit consent). Faresay does not determine the purposes of the clinical processing.

Draft prepared to support a UK launch. To be completed (placeholders) and validated by a data-protection solicitor.

Data Protection Impact Assessment (Art 35)

⚠️ DRAFT v0.1 — template for professional sign-off. NOT legal advice. Risk scoring to be validated by the DPO/solicitor. Last updated: 25 June 2026

Faresay Data Protection Impact Assessment (DPIA)

Prepared under **Article 35 of the UK GDPR**, following the ICO's DPIA methodology. A DPIA is **required** here because the processing involves **large-scale processing of special-category (health) data** using new technology — two of the ICO's screening triggers.

Assessment owner	Joel Harvey, Founder
DPO / data-protection lead	Joel Harvey, Founder (no statutory DPO required at current scale)
Date started / completed	25 June 2026
Review date	At least annually, or on material change

Step 1 — Identify the need for a DPIA

Faresay is a practice-management platform that therapists subscribe to in order to run their practice: managing clients, booking and delivering (video) sessions, taking card payment, and sending email/SMS reminders. Faresay acts as **Processor** for the clinical/client data; the therapist is the **Controller**.

A DPIA is required because the processing involves: (a) **special-category health data**; (b) at **scale** as the platform grows; (c) **vulnerable data subjects** (people seeking mental-health support); and (d) **new technological solutions** (video, automated reminders).

Step 2 — Describe the processing

Nature: collection, storage, transmission, display, payment facilitation, video hosting, messaging, automated reminders, export and deletion.

Scope: identity/contact data, scheduling data, payment metadata, messages, intake/consent responses, and **mental-health information**. Data subjects are clients of subscribing therapists (and the therapists themselves). Volume grows with adoption.

Context: clients are often vulnerable; there is a clear power/confidentiality dynamic in therapy. Data is among the most sensitive categories that exist. Faresay is UK-first.

Purposes: to let independent therapists deliver and administer care efficiently. Faresay's purpose as Processor is limited to providing the software on the Controller's instructions.

Data flows: client → therapist's practice on Faresay → stored in UK/EEA database (Neon); payments via Stripe; video via Daily.co; email via Resend; SMS via Twilio; cookieless analytics via Plausible. See [Sub-processor List](#).

Step 3 — Consultation

- **Data subjects / therapists:** To be recorded during design-partner onboarding.
- **Internal:** founder/engineering; DPO/solicitor on finalisation.
- **Processors:** reliance on sub-processors' published security/DPA documentation.

Step 4 — Assess necessity and proportionality

- **Lawful basis (as Controller, own data):** contract, legal obligation, legitimate interests (see [ROPA](#)).
- **Lawful basis (clinical data, as Processor):** determined by the therapist-Controller — typically Art 9(2)(h) and/or explicit consent.
- **Data minimisation:** only data needed to run a practice is collected; health data is recorded at the therapist's discretion; reminders contain practitioner + time, not clinical content; analytics are cookieless.
- **Function creep prevention:** Faresay contractually undertakes not to use client data for its own purposes, marketing, or AI training (DPA §3.3).
- **Data-subject rights:** supported via in-product export/deletion and Processor assistance to the Controller (DPA §7).

Step 5 — Identify and assess risks

#	Risk to individuals	Likelihood	Severity	Overall
R1	Unauthorised access to health data (external attacker)	Possible	Severe	High
R2	Therapist account compromise (weak/stolen credentials)	Possible	Severe	High
R3	A therapist sees another therapist's clients (authorisation flaw)	Remote	Severe	Medium
R4	Data exposure via a sub-processor / international transfer	Remote	Severe	Medium
R5	Reminder sent to wrong recipient (email/SMS)	Possible	Moderate	Medium
R6	Excessive retention after a client leaves a practice	Possible	Moderate	Medium
R7	Re-identification via analytics	Remote	Moderate	Low

Step 6 — Measures to reduce risk

#	Measure	Effect	Residual
R1	TLS in transit; AES-256 at rest; least-privilege access; security logging; pen-test pre/post scaled launch [target]	Reduce	Low–Medium
R2	Clerk auth with strong passwords + MFA ; session controls	Reduce	Low–Medium
R3	Application-level tenant isolation; authorisation checks on every client/session route; code review	Reduce	Low
R4	UK/EEA primary storage; UK IDTA / SCCs for any transfer; sub-processor DPAs; 14-day change notice	Reduce	Low
R5	Confirmed contact fields; managed-client flows; reminders carry minimal content	Reduce	Low

#	Measure	Effect	Residual
R6	Retention policy: export window + deletion timeline (DPA §11); per-client deletion	Reduce	Low
R7	Cookieless analytics (Plausible) — no personal data collected	Eliminate	Negligible

Step 7 — Sign-off and outcomes

Item	Detail
Residual risk	Acceptable — residual risks assessed Low after the measures above
Prior consultation with ICO required?	Only if a high residual risk cannot be mitigated — not anticipated .
DPO advice	to be confirmed
Approved by	Joel Harvey, Founder — 25 June 2026
Integrated into project?	Measures tracked in the Risk Register and TOMs .
Next review	to be confirmed

Draft prepared to support a UK launch. Risk scoring and sign-off to be validated by the DPO / data-protection solicitor before reliance.

Security & Data Protection Policy

⚠️ DRAFT v0.1 — for security + legal review. NOT legal advice. Must be validated before use. Last updated: 26 June 2026

Faresay Security & Data Protection Policy

Faresay operates an online therapy / mental-health marketplace that connects clients with licensed mental-health professionals. Because the Platform handles **mental-health information** — among the most sensitive categories of personal data that exist — Faresay applies a correspondingly high standard of security and data protection across its people, processes and technology.

This policy sets out the controls Faresay maintains to protect the confidentiality, integrity and availability of the data it processes. It is written to support:

- **United Kingdom (now):** the **UK GDPR** (in particular **Article 32 — Security of processing**) and the **Data Protection Act 2018**; clinical data is **special-category** health data; Faresay is registered with the **ICO**.
- **United States (planned expansion, all 50 states + DC):** the **HIPAA Security Rule** (45 CFR Part 160 and Part 164, Subparts A and C) and the **HIPAA Breach Notification Rule**, plus state health-privacy and consumer-protection laws (e.g. Washington **My Health My Data Act**, California **CCPA/CPRA**).

This policy should be read alongside the [Privacy Policy](#), the [Terms of Service](#), the [Therapist Agreement](#), and the shared [CONTEXT.md](#) fact sheet. It is an internal operational policy; client-facing commitments live in the Privacy Policy.

⚠️ **COUNSEL / SECURITY — overarching note.** This is a first draft for review. Faresay is **bootstrapped** and early-stage; several controls below describe a **target state** rather than a control that is fully implemented today. Each section flags where this is the case. Counsel and a security specialist must validate scope, the UK/EU vs US data-residency model, the controller / processor / covered-entity / business-associate analysis, and all breach-notification obligations before this policy is relied upon.

1. Purpose & scope

1.1 Purpose

This policy defines how Faresay protects the data it processes, establishes accountability for security and data protection, and demonstrates — for UK GDPR Article 32 and, on US launch, the HIPAA Security Rule — that Faresay implements appropriate technical and organisational measures (TOMs) proportionate to the risk to individuals.

1.2 Scope

This policy applies to: - All Faresay personnel: employees, founders, contractors, and any worker with access to Faresay systems or data. - All Faresay information systems and services, including the **marketplace application**, supporting infrastructure, code repositories, administrative tooling, and corporate accounts (email, collaboration tools). - All data processed by Faresay, with **mental-health data / Protected Health Information (PHI)** treated as the highest-sensitivity class (Section 2). - Third parties and sub-processors that process Faresay data on Faresay's behalf (Section 9).

1.3 Role boundary

Faresay provides the **platform**; the **clinician** provides the clinical service and owns the clinical relationship and clinical record (see [CONTEXT.md](#) and the [Therapist Agreement](#)). The split of data-protection roles — controller / processor / joint controller (UK/EU) and covered entity / business associate (US HIPAA) — is being finalised by counsel and affects which obligations in this policy fall on Faresay versus the clinician. ⚠️ **COUNSEL**.

2. Data classification

Faresay classifies data so that controls are applied proportionately. Mental-health data is always treated at the highest level.

Class	Examples	Handling baseline
Class 1 — Highest sensitivity: mental-health data / PHI / special-category health data	The fact a person is seeking or receiving therapy; intake/assessment information; session-related clinical information and notes; safeguarding/crisis information; messages relating to care.	Strict least-privilege access; encryption in transit and at rest; full audit logging; no use in non-production environments; breach-notification regimes apply (Section 14).
Class 2 — Confidential	Account & identity data; authentication data; payment-related data; clinician licensing/verification data; internal financials; security configuration.	Role-based access; encryption in transit and at rest; logged access.
Class 3 — Internal	Internal documents, non-sensitive operational data.	Access limited to personnel; standard controls.
Class 4 — Public	Marketing pages, published clinician profile information the clinician has agreed to publish.	Integrity controls; no confidentiality requirement.

Notes: - **Mental-health data is inherently sensitive**. Even the bare fact that an individual is a Faresay client is Class 1 data and must be protected accordingly. - Where Faresay acts as a **processor / business associate** for clinical records, the clinician (as controller / covered entity) sets the lawful basis and permitted uses; Faresay processes only on documented instructions. ⚠️ **COUNSEL**. - Payment card data: Faresay intends to use a **PCI-DSS-compliant payment processor** so that Faresay does not store raw card data. [PLACEHOLDER: payment processor]. ⚠️ **SECURITY** — confirm cardholder-data flows and PCI scope.

3. Governance, roles & responsibilities

3.1 Accountability

Faresay's leadership (founder/management) is ultimately accountable for information security and data protection and for approving this policy.

3.2 Key roles

- **Security Lead** — [PLACEHOLDER: named individual]. Owns this policy, the risk treatment plan, vendor security review, incident response coordination, and the security roadmap. In an early-stage bootstrapped company this

may initially be a founder; ⚠️ **SECURITY** — confirm whether a fractional/virtual CISO or external security advisor is engaged.

- **Data Protection Officer (DPO) / privacy lead** — Joel Harvey, Founder — support@faresay.com. ⚠️ **COUNSEL** — confirm whether a statutory **DPO is required** under UK GDPR Art 37 (likely triggered by large-scale processing of special-category health data) and, if so, appoint and register the DPO with the ICO. The DPO/privacy lead owns DPIAs, records of processing (Art 30), data-subject rights handling, and the ICO relationship.
- **HIPAA Security Official & Privacy Official (US)** — ⚠️ **COUNSEL** — the HIPAA Security Rule requires a designated **Security Official** (45 CFR §164.308(a)(2)) and the Privacy Rule a **Privacy Official**; these must be named before US launch if Faresay is a covered entity or business associate.
- **All personnel** — responsible for following this policy, completing training, reporting incidents promptly, and protecting credentials and devices.

3.3 Decision-making & review

Security risks are tracked in the [risk-register.md](#). Material security decisions, exceptions and accepted risks are recorded with an owner and review date.

4. Access control

4.1 Principles

- **Least privilege** — personnel receive the minimum access required for their role; broad/admin access is the exception, justified and time-bound where possible.
- **Role-Based Access Control (RBAC)** — access is granted by role, not individually, wherever the platform and tooling support it.
- **Need-to-know for Class 1 data** — access to mental-health data / PHI is restricted to roles with a genuine operational need and is logged (Section 11).
- **Separation of duties** — production access, code-deployment rights and security administration are separated where headcount allows; ⚠️ **SECURITY** — document compensating controls given small team size.

4.2 Authentication

- **Multi-factor authentication (MFA)** is required for all administrative and production-system access, code repositories, the hosting provider, DNS, email, and any system holding Class 1 or Class 2 data.
- Strong, unique passwords managed via a password manager; no shared accounts for administrative access.
- ⚠️ **SECURITY** — adopt SSO with enforced MFA across corporate SaaS where feasible. [PLACEHOLDER: identity provider / SSO].

4.3 Joiner / mover / leaver (JML)

- **Joiner** — access provisioned by role on a documented request; principle of least privilege; security onboarding and confidentiality agreement before access to Class 1/Class 2 data.
- **Mover** — access reviewed and adjusted on role change; entitlements no longer needed are revoked.
- **Leaver** — all access revoked promptly on departure (target: same business day; immediately for involuntary departures); credentials rotated; devices returned/wiped.

- **Periodic access reviews** — entitlements (especially to Class 1 data and admin roles) reviewed at least [PLACEHOLDER: quarterly] and stale access removed.
-

5. Encryption & key management

5.1 In transit

- **TLS** (current secure version, [PLACEHOLDER: TLS 1.2+ / 1.3]) enforced for all connections to the Platform and between services. HTTP redirected to HTTPS; HSTS enabled.
- TLS termination and edge security are provided via **Cloudflare** (DNS) and **Vercel** (web hosting) for the marketplace web app. ⚠️ **SECURITY** — confirm certificate management and that all backend/API and database connections are also encrypted.

5.2 At rest

- Class 1 and Class 2 data encrypted at rest using strong, industry-standard algorithms (e.g. AES-256). [PLACEHOLDER: database / storage provider and their at-rest encryption].
- Backups encrypted at rest (Section 13).

5.3 Key management

- Encryption keys and secrets (API keys, database credentials) managed via a dedicated secrets manager / platform environment-variable store, never committed to source control. [PLACEHOLDER: secrets management solution].
 - Key access restricted, logged, and keys rotated on a defined schedule and on suspected compromise. ⚠️ **SECURITY** — define key-rotation cadence and ownership.
-

6. Infrastructure, hosting & data residency

6.1 Known stack (per CONTEXT.md)

- **Marketplace application** — the core product.
- **Web hosting: Vercel.**
- **DNS: Cloudflare.**
- **Application database / backend services / file storage:** [PLACEHOLDER: provider(s) — not specified in CONTEXT]. ⚠️ **SECURITY** — document the full inventory: database, object storage, email, payments, video/session delivery, analytics, error monitoring.

6.2 Data residency ⚠️ COUNSEL

- **UK/EU (now):** confirm that personal data — especially Class 1 mental-health data — is stored and processed in the **UK/EU**, or that a lawful **international transfer** mechanism is in place (UK IDTA / Addendum, EU SCCs, adequacy). Vercel and Cloudflare operate globally; **region configuration and routing must be verified**, since default configurations may route or store data outside the UK/EU. ⚠️ **COUNSEL.**
- **US (planned):** determine US data-residency requirements and whether US client data must remain in the US. Confirm that any provider handling PHI will sign a **HIPAA Business Associate Agreement** and offers a HIPAA-

eligible configuration (Section 9). ⚠️ **COUNSEL**.

- ⚠️ **SECURITY** — maintain a data-flow map showing where each data class is stored, processed and transmitted, including sub-processor locations.

7. Secure software development lifecycle (SSDLC)

- **Secure by design** — security and privacy considered from design; a **DPIA** is completed for high-risk processing of special-category data (UK GDPR Art 35). ⚠️ **COUNSEL**.
- **Source control & review** — code held in version control ([PLACEHOLDER: GitHub/GitLab]); changes via pull request with **peer review** before merge to production where headcount allows; protected main branch.
- **Secrets hygiene** — secret scanning enabled; no credentials in code or logs.
- **Dependency & supply-chain security** — automated dependency scanning (e.g. Dependabot/equivalent) and timely patching of vulnerable libraries.
- **Application security** — input validation, output encoding, parameterised queries, authentication/authorisation checks, protection against OWASP Top 10 risks.
- **Environment separation** — production, staging and development environments separated; **production Class 1 data is never used in non-production environments** (use synthetic/anonymised test data).
- **CI/CD** — automated build, test and deploy pipelines with access controls on deployment. [PLACEHOLDER: CI/CD tooling].
- ⚠️ **SECURITY** — formalise security testing (SAST/DAST) in the pipeline as the team matures.

8. (Reserved — see Section 9 for third-party management)

Section intentionally consolidated into Section 9.

9. Third-party & sub-processor management

9.1 Inventory & due diligence

- Faresay maintains a register of all sub-processors / vendors that process Faresay data, the data classes involved, and their locations. [PLACEHOLDER: sub-processor register].
- Vendors handling Class 1/Class 2 data are subject to security due diligence proportionate to risk (e.g. SOC 2 / ISO 27001 evidence, security questionnaire).

9.2 Contracts — DPAs and BAAs ⚠️ **COUNSEL**

- **UK/EU**: a **Data Processing Agreement (DPA)** compliant with UK GDPR Art 28 is required with every processor handling personal data, including appropriate international-transfer terms (IDTA/SCCs).
- **US / HIPAA**: where Faresay is a **covered entity or business associate** and a vendor will create, receive, maintain or transmit **PHI**, a **HIPAA Business Associate Agreement (BAA)** is **mandatory** before any PHI is shared. This includes hosting, storage, email, video/session, and analytics providers that touch PHI. ⚠️ **COUNSEL**.

- **⚠ SECURITY / COUNSEL** — confirm which of the current providers (Vercel, Cloudflare, and the to-be-confirmed database/email/payment/video providers) will sign a BAA and offer a HIPAA-eligible plan; some default plans do **not** include a BAA. Do not transmit PHI to any provider without a signed BAA.
-

10. (Reserved)

See Section 11.

11. Logging, monitoring & audit trails

- **Audit logging** — access to and changes affecting Class 1 (mental-health/PHI) and Class 2 data are logged with user identity, timestamp and action, to support HIPAA audit-control requirements (45 CFR §164.312(b)) and UK GDPR accountability. **⚠ SECURITY** — confirm application-level audit logging covers reads of clinical data, not just writes.
 - **Security monitoring** — infrastructure, authentication and application logs collected and reviewed; alerting on suspicious activity (e.g. failed-login spikes, privilege changes). [PLACEHOLDER: logging/monitoring/SIEM tooling].
 - **Log protection & retention** — logs protected against tampering and unauthorised access, and retained for [PLACEHOLDER: retention period — align with HIPAA's 6-year documentation retention and ICO guidance]. **⚠ COUNSEL.**
 - **No sensitive data in logs** — Class 1 content and secrets must not be written to application logs.
-

12. Vulnerability management & penetration testing

- **Patch management** — operating systems, dependencies and platform components kept up to date; critical vulnerabilities remediated on a defined SLA (target: [PLACEHOLDER: e.g. critical within 7 days]).
 - **Vulnerability scanning** — regular automated scanning of dependencies and, where applicable, infrastructure.
 - **Penetration testing** — independent penetration test of the Platform before US launch and at least annually / on major change thereafter. [PLACEHOLDER: pentest provider/cadence]. **⚠ SECURITY.**
 - **Coordinated disclosure** — a channel for reporting vulnerabilities ([PLACEHOLDER: security@ address]) and a triage process.
-

13. Backup, disaster recovery & business continuity

- **Backups** — regular automated backups of production data, encrypted at rest, with backup integrity/restore testing. [PLACEHOLDER: backup solution, frequency, retention].
 - **Recovery objectives** — defined **RPO** and **RTO**: [PLACEHOLDER: RPO/RTO targets]. **⚠ SECURITY.**
 - **Disaster recovery** — documented procedure to restore service after major failure; reliance on managed providers (Vercel, Cloudflare, and the database provider) is noted, with their resilience/SLA considered.
 - **Business continuity** — plan for continuity of essential operations (including client safety and crisis pathways — see [crisis-safeguarding-policy.md]) during disruption. **⚠ CLINICAL / SECURITY.**
-

14. Incident response & breach notification ⚠️ COUNSEL

14.1 Incident response

- A documented incident-response process covers detection, triage, containment, eradication, recovery, notification and post-incident review.
- All personnel must report suspected incidents immediately to the Security Lead. [PLACEHOLDER: reporting channel].
- Incidents are logged and assessed for severity and for whether a notifiable personal-data breach has occurred.

14.2 UK breach notification

- Where a personal-data breach is likely to result in a risk to individuals' rights and freedoms, Faresay (or the responsible controller) notifies the **ICO without undue delay and within 72 hours** of becoming aware (UK GDPR Arts 33–34).
- Where the breach is likely to result in a **high risk**, affected individuals are notified without undue delay.
- ⚠️ **COUNSEL** — confirm Faresay's notification role given the controller/processor split; a processor must notify the controller without undue delay rather than the regulator directly.

14.3 US breach notification (on US launch)

- The **HIPAA Breach Notification Rule** (45 CFR §§164.400–414) requires, for breaches of unsecured PHI, notification to affected individuals (generally within **60 days**), to **HHS**, and — for larger breaches — to the media; business associates must notify the covered entity.
 - **State Attorney General / state breach-notification laws** impose additional and varying obligations across the 50 states + DC (timing, content, AG notice), and consumer-health-data laws (e.g. Washington **My Health My Data Act**) may add further requirements.
 - ⚠️ **COUNSEL** — build a state-by-state breach-notification matrix before US launch; obligations and deadlines vary materially by state.
-

15. Personnel security

- **Background checks** — pre-engagement screening proportionate to role and access to Class 1 data, subject to local law (e.g. UK DBS where appropriate; US background checks). ⚠️ **COUNSEL** — confirm lawful basis/limits for screening in each jurisdiction. (Clinician licensing/verification is handled separately under the [Therapist Agreement](#) and clinical governance.)
 - **Confidentiality agreements** — all personnel with access to Class 1/Class 2 data sign confidentiality / NDA terms.
 - **Security & data-protection training** — at onboarding and at least annually, including phishing awareness and handling of mental-health data; HIPAA workforce training before US launch (45 CFR §164.308(a)(5)).
 - **Sanctions** — defined consequences for policy violations.
-

16. Physical security

- Faresay is a [PLACEHOLDER: remote-first?] organisation relying primarily on **cloud-hosted** infrastructure; physical data-centre security is inherited from providers (Vercel, Cloudflare, and the database provider) and evidenced via their certifications (e.g. SOC 2 / ISO 27001).
 - **Endpoint security** — personnel devices accessing Faresay data must use full-disk encryption, screen lock, current OS/security updates, and reputable endpoint protection; lost/stolen devices reported immediately.
 - **Workspace** — Class 1 data must not be viewed in insecure public settings; clear-screen/clear-desk discipline applies.
 -  **SECURITY** — confirm any office/physical access controls and mobile-device management (MDM). [PLACEHOLDER: MDM solution].
-

17. Acceptable use

- Faresay systems and data are used only for authorised business purposes.
 - No sharing of credentials; no exporting or copying Class 1 data outside approved systems; no use of unapproved tools/services (including AI tools) to process Class 1/Class 2 data without authorisation.
 - Personnel must protect devices and credentials, follow this policy, and report security concerns.
 -  **SECURITY** — define an approved-tools list and rules for any AI/LLM tooling that could expose mental-health data.
-

18. Mapping to HIPAA Security Rule safeguards **COUNSEL**

The following maps this policy to the HIPAA Security Rule safeguard categories. This is a **planning aid for US launch**, not a confirmation of compliance, and assumes Faresay is a covered entity and/or business associate (to be confirmed).  **COUNSEL**.

18.1 Administrative safeguards (45 CFR §164.308)

- Security Management Process / risk analysis & risk management — Sections 3, 12; [risk-register.md].
- Assigned Security Responsibility (Security Official) — Section 3.  **COUNSEL** (appoint).
- Workforce Security & Information Access Management — Sections 4, 15.
- Security Awareness & Training — Section 15.
- Security Incident Procedures — Section 14.
- Contingency Plan (backup, DR, emergency mode) — Section 13.
- Evaluation — Section 21 (periodic review).
- Business Associate Contracts — Section 9 (BAAs).

18.2 Physical safeguards (45 CFR §164.310)

- Facility Access Controls — Section 16 (inherited from providers).
- Workstation Use & Security — Sections 16, 17.
- Device & Media Controls (disposal, re-use, accountability) — Sections 16, 19.

18.3 Technical safeguards (45 CFR §164.312)

- Access Control (unique user ID, automatic logoff, encryption/decryption) — Sections 4, 5.
- Audit Controls — Section 11.
- Integrity — Sections 5, 7, 11.
- Person/Entity Authentication — Section 4 (MFA).
- Transmission Security — Section 5 (TLS).

⚠️ **COUNSEL / SECURITY** — a full HIPAA Security Rule gap assessment, risk analysis and documentation set must be completed before US launch.

19. Data retention, minimisation & secure disposal

- **Minimisation** — Faresay collects and retains only the data necessary for the purposes set out in the [Privacy Policy](#).
- **Retention** — data retained per a defined retention schedule. Note that **clinical records are typically owned and retained by the clinician** under professional/clinical-records rules; Faresay's retention of platform data is separate. [PLACEHOLDER: retention schedule]. ⚠️ **COUNSEL / CLINICAL**.
- **Secure disposal** — data securely deleted/anonymised at end of retention; media sanitised or providers' certified-destruction processes relied upon; backups age out per the backup retention policy.
- ⚠️ **COUNSEL** — reconcile retention with UK GDPR storage limitation, HIPAA's 6-year documentation retention, and clinical-records retention rules, which differ.

20. Alignment to a recognised framework (future goal)

Faresay's security programme is structured to align over time with a recognised framework. As a **bootstrapped, early-stage** company, formal certification is a **future goal** rather than a current state. Candidate frameworks: - **SOC 2 (Type II)** — likely the most market-relevant signal for a US healthtech marketplace and enterprise/clinician trust. - **ISO/IEC 27001** — internationally recognised ISMS certification. - **NIST Cybersecurity Framework / NIST 800-53** — useful as a control reference.

⚠️ **SECURITY** — agree the target framework and a realistic roadmap (likely: implement controls now, pursue SOC 2 around US launch).

21. Policy review cadence

- This policy is reviewed at least **annually** and after any major change to systems, processing, regulation, or following a significant incident.
- The Security Lead owns the review; the DPO/privacy lead and counsel review data-protection and regulatory aspects.
- Version history and review dates are maintained below.

Version	Date	Author	Notes
v0.1 (DRAFT)	26 June 2026	[PLACEHOLDER: author]	Initial draft for security + legal review.

End of draft. ⚠️ This document is a v0.1 draft and must be validated by qualified security and legal professionals, with all [PLACEHOLDER: ...] items resolved, before it is relied upon or published.

Clinical Governance Policy

⚠️ DRAFT v0.1 — for clinical advisor + legal review. NOT clinical or legal advice. Must be validated by a qualified clinical lead before use. Last updated: 26 June 2026

Faresay Clinical Governance Policy

1. Purpose and Scope

1.1 Purpose

1.1.1 This Clinical Governance Policy (the "**Policy**") sets out the framework through which Faresay seeks to assure the quality and safety of the clinical services delivered by independent mental-health professionals through the Faresay marketplace platform.

1.1.2 The Policy describes how clinical standards are set, how practitioners are verified and monitored, how clinical quality and safety are overseen, and how clinical risks, incidents, and complaints are managed. Its objective is to support safe, ethical, evidence-informed care while preserving each practitioner's independent professional clinical judgement.

1.1.3 Faresay provides the **technology and marketing platform** (software, discovery, scheduling, payments, client acquisition, and administrative support). Faresay does **not** provide clinical services, does not employ practitioners to provide clinical services, and is not a party to the clinical relationship. The practitioner provides the clinical service and owns the clinical relationship and the clinical record. This Policy is a governance and quality-assurance framework for the marketplace; it is not, and must not be read as, Faresay directing, controlling, or interfering with any practitioner's clinical judgement.

⚠️ COUNSEL: The boundary between (a) legitimate marketplace quality/safety governance and (b) the practice of psychology/medicine or the exercise of clinical control matters for corporate-practice-of-medicine / corporate-practice-of-psychology (CPOM) doctrine, fee-splitting rules, and the MSO / "friendly PC" structure. The whole Policy must be reviewed by US healthcare-regulatory counsel (per state) and UK counsel to confirm it does not, in substance or appearance, transfer clinical control to a non-clinical entity. See `therapist-agreement.md` clauses 2.4–2.6.

1.2 Scope

1.2.1 This Policy applies to:

- all mental-health practitioners onboarded to and delivering services through the Platform ("Practitioners");
- Faresay personnel and contractors involved in clinical governance, onboarding, verification, trust and safety, and support;
- the clinical advisory function described in Section 3;
- where applicable in a US state, any professional entity ("friendly PC" / professional corporation) through which clinical services are contracted or supervised.

1.2.2 **Territory.** This Policy applies to Faresay's current operations in the **United Kingdom** and to its planned expansion into the **United States (the 50 states and the District of Columbia)**. Where a requirement differs

between the UK and the US, or between US states, this is flagged. US-specific requirements take effect only on launch in the relevant state and are subject to validation by counsel and the clinical lead.

1.2.3 This Policy sits alongside and should be read with: the Therapist Agreement ([therapist-agreement.md](#)), the Crisis / Safeguarding Policy ([crisis-safeguarding-policy.md](#)), the Privacy Policy ([privacy-policy.md](#)), the Security & Data Protection Policy ([security-data-protection-policy.md](#)), and the Terms of Service ([terms-of-service.md](#)).

1.3 Status and ownership of this Policy

1.3.1 This Policy is owned by the Clinical Lead (see Section 3) and approved by [PLACEHOLDER: approving body — e.g. Faresay board / clinical advisory committee].

1.3.2 ⚠️ CLINICAL: This draft has been prepared without a qualified clinical lead in post. Every clinical standard, threshold, and exclusion in this Policy must be reviewed, corrected, and validated by a qualified clinical lead (and, where US-specific, by US-licensed clinical advisors) before the Policy is relied upon.

2. Definitions

In this Policy:

- **"Client"** means an individual who books, accesses, or receives clinical services from a Practitioner via the Platform.
- **"Clinical Lead"** means the qualified, suitably-registered senior clinician accountable for clinical governance at Faresay, as described in Section 3.
- **"Clinical Advisory Function"** means the clinical advisory committee, panel, or named advisors supporting the Clinical Lead, as described in Section 3.
- **"Clinical Record"** means the clinical notes, assessments, treatment plans, correspondence, and other professional records a Practitioner creates in connection with the care of a Client.
- **"Clinical Services"** means the regulated mental-health, psychotherapy, or counselling services a Practitioner provides to Clients.
- **"Compact"** means an interstate licensure compact relevant to mental-health practice, including PSYPACT (psychology), the Counseling Compact, and the Social Work Licensure Compact.
- **"DBS"** means the UK Disclosure and Barring Service.
- **"Friendly PC" / "PC"** means a US professional corporation or professional entity through which clinical services may be contracted or supervised in a corporate-practice state.
- **"MSO"** means a management services organisation (Faresay's commercial / non-clinical entity in a US corporate-practice structure).
- **"Practitioner"** means an independent, licensed mental-health professional onboarded to deliver Clinical Services through the Platform.
- **"Professional Body"** means the relevant statutory regulator or professional membership/accreditation body for a Practitioner (see Section 18).
- **"Routine Outcome Measure" / "ROM"** means a validated, standardised measure used to track client symptoms, functioning, or progress over the course of care.
- **"Scope of Practice"** means the range of clinical activities a Practitioner is qualified, trained, competent, and licensed/registered to perform.
- **"State"**, in the US context, means a US state or the District of Columbia.

- **"Telehealth"** means the delivery of Clinical Services remotely by video, voice, or messaging.
-

3. Governance Structure and Accountability

3.1 Principle of clinical leadership

3.1.1 ⚠️ CLINICAL: Clinical governance at Faresay must be led by a qualified clinician, not by commercial or product staff. Faresay will appoint a **Clinical Lead** who is professionally responsible and accountable for the matters in this Policy.

3.1.2 The Clinical Lead must hold a current, valid, unrestricted licence/registration with a relevant Professional Body and have appropriate seniority and experience in mental-health practice and clinical governance.

[PLACEHOLDER: named Clinical Lead, qualifications, registration number.]

⚠️ COUNSEL / ⚠️ CLINICAL: In US corporate-practice states, the senior clinical authority may need to sit within (or be appointed by) the professional entity ("friendly PC"), not the MSO, to avoid the appearance of a non-clinical entity controlling clinical decisions. The reporting line and appointment mechanism for the Clinical Lead in the US structure must be designed with counsel.

3.2 Role of the Clinical Lead

3.2.1 The Clinical Lead is responsible for: owning and maintaining this Policy and related clinical standards; overseeing Practitioner onboarding, verification, and credentialing criteria (Section 4); defining scope-of-practice and exclusion criteria (Sections 6 and 15); overseeing quality assurance and outcome monitoring (Section 11); chairing or convening the Clinical Advisory Function; overseeing clinical incident review (Section 12) and the clinical complaints route (Section 13); and advising the business on clinical risk.

3.2.2 The Clinical Lead may delegate operational tasks but remains accountable for the clinical governance framework.

3.3 Clinical Advisory Function

3.3.1 ⚠️ CLINICAL: Faresay will establish a **Clinical Advisory Function** — a committee, panel, or set of named advisors — to support the Clinical Lead. Its composition, terms of reference, quorum, and meeting cadence are to be defined. [PLACEHOLDER: committee structure and members.]

3.3.2 The Clinical Advisory Function should, at minimum, include clinicians representing the main professional disciplines on the Platform (e.g. clinical/counselling psychology, psychotherapy, counselling, clinical social work) and, on US expansion, US-licensed clinicians familiar with the relevant state and professional standards. ⚠️ CLINICAL: confirm the appropriate disciplinary mix and whether independent (external) advisors are required.

3.3.3 Indicative responsibilities: reviewing and endorsing clinical standards and exclusion criteria; periodic case review and audit (Section 11); reviewing serious clinical incidents and trends (Section 12); and advising on emerging clinical risks and evidence.

3.4 Non-clinical roles

3.4.1 Faresay's trust-and-safety, onboarding/verification, support, data-protection, and engineering teams support clinical governance operationally (for example, by running verification checks and maintaining systems) but do not make clinical decisions. Clinical decisions about individual clients rest solely with the treating Practitioner.

3.4.2 A roles-and-responsibilities matrix is set out in Section 17.

4. Practitioner Onboarding and Verification

4.1 Principle

4.1.1 No Practitioner may deliver Clinical Services through the Platform until they have completed onboarding and verification and have been approved. Onboarding establishes that a Practitioner is appropriately qualified, licensed/registered, insured, and fit to practise.

4.2 Licensure / registration verification

4.2.1 **UK.** Faresay will verify that the Practitioner holds a current, valid, unrestricted registration or accredited membership with the relevant UK Professional Body (e.g. HCPC for practitioner psychologists; BACP, UKCP, or BABCP for counsellors/psychotherapists, as applicable). ⚠️ **CLINICAL:** confirm the minimum acceptable registers/accreditations Faresay will accept, and whether unaccredited counsellors are excluded.

4.2.2 **US — licence in the client's state.** A Practitioner must hold a current, valid, unrestricted licence to practise in the **State in which the Client is physically located at the time of the session**, consistent with the licensing requirements of that State's board. Faresay must verify licensure against the relevant State licensing board for each State in which the Practitioner is offered to clients.

⚠️ **COUNSEL / ⚠️ CLINICAL:** Telehealth licensure generally turns on where the *client* is located, not the practitioner. Per-State rules, exceptions, and any temporary/telehealth registrations must be confirmed by counsel and reflected in the platform's matching logic so a Practitioner can only be booked by clients in States where they are authorised.

4.2.3 **Interstate compacts.** Where a Practitioner holds authority under an applicable Compact, Faresay may verify and rely on that authority for the relevant States, subject to the compact's terms:

- **PSYPACT** for qualifying psychologists;
- the **Counseling Compact** for qualifying professional counsellors;
- the **Social Work Licensure Compact** for qualifying clinical social workers.

⚠️ **COUNSEL:** Compact participation, the scope of authority each compact grants (e.g. telehealth vs temporary in-person practice), member-State status, and the home-State requirement vary and change over time. Reliance on a compact for any given Client/State must be validated by counsel and re-checked as compacts and State participation evolve.

4.2.4 Verification must capture and store, at minimum: the licensing/registering body, licence/registration number, jurisdiction(s), licence type and scope, issue and expiry dates, and current standing. [PLACEHOLDER: verification provider / method — e.g. primary-source verification vendor.]

4.3 References

4.3.1 Faresay will obtain and review [PLACEHOLDER: number] professional reference(s) appropriate to the role. ⚠️ **CLINICAL:** confirm reference requirements (number, source, whether clinical-supervisor references are required).

4.4 Background / DBS checks

4.4.1 **UK.** Faresay will require an appropriate criminal-records / suitability check. ⚠️ **COUNSEL / ⚠️ CLINICAL:** confirm the correct DBS level for remote, non-regulated-activity mental-health work (basic vs standard vs enhanced), and whether the work meets the definition of "regulated activity" — this affects DBS eligibility and barred-list checks. Do not assert a DBS level until confirmed.

4.4.2 **US.** Faresay will require an appropriate background check consistent with applicable State and federal law. ⚠️ **COUNSEL:** background-check scope, consent (FCRA), and use of results vary by State and are subject to fair-chance / "ban-the-box" and adverse-action rules; confirm with counsel before defining the standard.

4.5 Professional indemnity insurance

4.5.1 Each Practitioner must hold and maintain professional indemnity / professional liability (malpractice) insurance appropriate to their discipline, jurisdiction(s) of practice, and the telehealth services delivered, at coverage limits no lower than [PLACEHOLDER: minimum limits]. Faresay will verify cover at onboarding and on renewal.

⚠️ **COUNSEL** / ⚠️ **CLINICAL:** Minimum coverage limits, whether telehealth and the relevant US States are within the policy's scope, and any requirement for Faresay to be named/additional insured must be set with counsel and the clinical lead. UK and US norms differ.

4.6 Fitness and declarations

4.6.1 At onboarding and on renewal, Practitioners must declare any past or pending fitness-to-practise proceedings, licence conditions/restrictions/suspensions, criminal matters, and material complaints or claims. Material non-disclosure may result in removal from the Platform.

4.7 Ongoing licence / sanction monitoring

4.7.1 Verification is **not** a one-time event. Faresay will operate ongoing monitoring of each Practitioner's continued eligibility, including:

- monitoring **licence/registration expiry** and requiring evidence of renewal before lapse;
- periodic re-verification of standing with each relevant Professional Body / State board on a defined cadence ([PLACEHOLDER: e.g. at least annually, and on renewal]);
- monitoring for **disciplinary actions, sanctions, conditions, suspensions, or restrictions**, including (US) checks against State board actions and relevant national databases. ⚠️ **COUNSEL:** confirm permissible/required sources (e.g. NPDB access eligibility, OIG/SAM exclusion lists, State board disciplinary feeds) — access to some databases is restricted to certain entity types.
- monitoring lapses or changes to professional indemnity insurance.

4.7.2 On discovery of an expiry, sanction, restriction, or loss of insurance affecting a Practitioner's eligibility for any State/jurisdiction, Faresay will suspend the Practitioner's availability for the affected jurisdiction(s) pending review by the Clinical Lead, and manage any client-continuity-of-care implications under Sections 6, 12, and the Crisis / Safeguarding Policy ([crisis-safeguarding-policy.md](#)).

5. Standards of Care and Evidence-Based Practice

5.1 Practitioners are expected to deliver care that is consistent with current, recognised, evidence-based or evidence-informed practice for the presenting need and client population, within the limits of telehealth delivery.

5.2 Practitioners must comply with the ethical and practice standards of their Professional Body / licensing board (Section 18), including standards specific to remote/telehealth delivery.

5.3 ⚠️ **CLINICAL:** Confirm whether Faresay will reference or require adherence to specific clinical guidelines (e.g. NICE guidance in the UK; recognised US professional-body practice guidelines) or remain at the level of

"consistent with the Practitioner's professional standards and competence." Avoid setting standards that could be read as Faresay directing clinical care.

5.4 Practitioners must keep their knowledge and skills current (see Section 10) and practise within their competence (Section 6).

6. Scope of Practice and Competence

6.1 Practising within competence

6.1.1 Practitioners must only provide Clinical Services that fall within their **Scope of Practice** — that is, services they are qualified, trained, competent, licensed/registered, and (where required) supervised to provide — and only with client populations and presentations they are competent to treat.

6.1.2 Practitioners must not work beyond their competence and must refer or signpost a Client elsewhere where the Client's needs fall outside the Practitioner's competence or outside what can be safely delivered on this Platform.

6.2 Presentations unsuitable for the Platform

6.2.1 ⚠️ CLINICAL: Faresay operates a **cash-pay telehealth marketplace**. Certain presentations and acuity levels are not appropriate for this setting and must be excluded from, or escalated out of, Platform-based care. These include (indicatively, to be finalised by the Clinical Lead):

- active suicidal or homicidal crisis, or imminent risk of serious harm to self or others;
- presentations requiring urgent, emergency, or in-person crisis intervention or hospitalisation;
- conditions requiring a level or intensity of care that telehealth cannot safely provide (e.g. acute psychosis requiring stabilisation, severe/high-risk eating disorders requiring medical monitoring, acute withdrawal/detoxification);
- circumstances where the Client cannot be safely treated remotely (e.g. no safe, private environment; jurisdictional/age limits).

6.2.2 The detailed clinical thresholds, screening, and the response when an excluded presentation is identified before or during care (including safe referral, escalation, and signposting to emergency services) are governed by the **Crisis / Safeguarding Policy ([crisis-safeguarding-policy.md](#))**, which this Policy cross-references and must be read with. ⚠️ CLINICAL: ensure the exclusion list here and the screening/escalation pathways in the Crisis / Safeguarding Policy are mutually consistent.

7. Informed Consent

7.1 Before Clinical Services begin, the Practitioner is responsible for obtaining the Client's informed consent to treatment, consistent with the Practitioner's professional and legal obligations.

7.2 Informed consent should cover, at minimum: the nature, format, and limits of telehealth delivery; the likely approach and any material risks/benefits and alternatives; confidentiality and its limits (including duty-to-warn / safeguarding disclosures — see [crisis-safeguarding-policy.md](#)); how clinical records are kept and by whom; fees and the cash-pay model; and arrangements for crisis/out-of-hours situations and what to do in an emergency.

7.3 ⚠️ CLINICAL / ⚠️ COUNSEL: Informed-consent content and documentation requirements differ by jurisdiction and discipline (e.g. specific US State telehealth-consent statutes; capacity and consent for minors; emergency-

contact and "patient location" attestations). Confirm jurisdiction-specific consent requirements and whether consent must be documented in a prescribed form.

7.4 Consent to **clinical** treatment (the Practitioner's responsibility) is distinct from consent to **data processing** by Faresay as a platform, which is addressed in the Privacy Policy (`privacy-policy.md`).

8. Clinical Record-Keeping Standards

8.1 The Practitioner / PC holds the clinical record

8.1.1 The **clinical record is owned and held by the Practitioner** (or, in a US corporate-practice structure, by the professional entity / "friendly PC" through which the Practitioner provides care) — **not by Faresay**. The Practitioner is the controller/custodian of the Clinical Record and is responsible for creating, maintaining, retaining, securing, and (where required) disposing of it in accordance with law and professional standards. See `therapist-agreement.md` clause 2.3.

8.1.2 Faresay processes Client and platform data as a technology provider as described in the Privacy Policy (`privacy-policy.md`) and Security & Data Protection Policy (`security-data-protection-policy.md`); this is distinct from holding the Clinical Record.

⚠ COUNSEL: The controller/processor (UK GDPR) and covered-entity/business-associate (US HIPAA) characterisation of Faresay versus the Practitioner/PC for the Clinical Record and associated data must be confirmed with counsel, and may require a data processing agreement / business associate agreement. See `security-data-protection-policy.md` .

8.2 Minimum record-keeping standards

8.2.1 Practitioners must keep clinical records that are, at minimum:

- **accurate, contemporaneous, and legible** — created at or close to the time of the contact;
- **adequate** — sufficient to support continuity and the clinical decisions made, including presenting issues, assessment, risk assessment, formulation/treatment plan, consent, session notes, and any referrals or safeguarding actions;
- **secure and confidential** — stored to the security standard required by law and `security-data-protection-policy.md` ;
- **retained** for the period required by the Practitioner's jurisdiction(s) and Professional Body, and then securely disposed of.

8.2.2 [PLACEHOLDER: minimum retention periods] ⚠ CLINICAL / ⚠ COUNSEL: retention periods differ by jurisdiction, discipline, and client age (e.g. records for minors). Confirm per UK and per US State, and whether Faresay's platform tooling stores any clinical content (and if so, under what agreement).

9. Clinical Supervision and Continuing Professional Development

9.1 Practitioners must maintain clinical supervision and continuing professional development (CPD) consistent with the requirements of their Professional Body / licensing board, and must be able to evidence this on request.

9.2 ⚠ CLINICAL: Some UK modalities/bodies require ongoing supervision (e.g. BACP's supervision requirement for counsellors); US requirements vary by State and licence type (including supervision requirements for

pre-/associate-licensed clinicians). Confirm the minimum supervision and CPD standards Faresay will require, and whether pre-/associate-licensed practitioners are admitted at all and, if so, how supervision is evidenced.

9.3 Faresay does not act as the Practitioner's clinical supervisor and does not provide clinical supervision; responsibility for arranging adequate supervision rests with the Practitioner.

10. Quality Assurance and Outcome Monitoring

10.1 ⚠️ CLINICAL: Faresay will operate a clinical quality-assurance programme, overseen by the Clinical Lead and Clinical Advisory Function. The detailed design — including which measures, thresholds, and review cadences are appropriate and clinically valid — must be set by the clinical lead. Indicative components:

- **Routine Outcome Measures (ROMs):** use of validated, standardised measures (e.g. [PLACEHOLDER: instruments — to be selected by clinical lead]) to track client progress, used as a clinical tool by Practitioners rather than as a performance metric imposed by Faresay.
- **Client feedback:** structured post-session/episode feedback and satisfaction measures, and a clear route for clients to raise clinical concerns (Section 13).
- **Periodic case review / clinical audit:** sampling and review of de-identified cases or anonymised practice data against agreed standards, by the Clinical Advisory Function.
- **Practitioner quality signals:** monitoring of indicators such as cancellation/no-attend patterns, complaints, and incidents, used supportively and proportionately.

10.2 ⚠️ COUNSEL / ⚠️ CLINICAL: Outcome and case-review activity must be designed so it does not (a) involve Faresay in the practice of psychology/medicine or override clinical judgement, or (b) breach client confidentiality / data-protection law. Confirm the lawful basis, de-identification approach, and consent position for any clinical-data use with counsel; confirm clinical validity with the clinical lead.

10.3 Quality-assurance findings feed into Practitioner support, this Policy's periodic review (Section 16), and the risk register (`risk-register.md`).

11. Incident Reporting and Management

11.1 A **clinical incident** is any event in connection with Clinical Services that caused, or had the potential to cause, harm to a Client or another person — including safeguarding concerns, serious adverse events, suspected boundary or ethical breaches, data incidents affecting clinical information, and "near misses."

11.2 Practitioners and Faresay personnel must report clinical incidents promptly through [PLACEHOLDER: reporting channel / form]. Faresay will maintain an incident log and triage incidents to the Clinical Lead.

11.3 The Clinical Lead (with the Clinical Advisory Function for serious incidents) will ensure each incident is reviewed, immediate client safety is addressed, root causes and learning are identified, and corrective actions are tracked to completion. Trends are reported into quality assurance (Section 10) and the risk register (`risk-register.md`).

11.4 ⚠️ CLINICAL / ⚠️ COUNSEL: Certain incidents trigger **external** reporting or notification duties — for example to a regulator/licensing board, a safeguarding authority, an insurer, or (for data breaches) the ICO/affected individuals (UK) or under HIPAA breach-notification rules (US). The mandatory external reporting matrix must be confirmed per jurisdiction with counsel and the clinical lead. Crisis events (e.g. risk to life) are governed by the Crisis / Safeguarding Policy (`crisis-safeguarding-policy.md`).

11.5 Faresay will not retaliate against good-faith reporting of incidents or concerns.

12. Complaints Handling — Clinical Route

12.1 Faresay operates a clear route for Clients (and others) to raise concerns or complaints. Complaints are triaged into:

- **Clinical complaints** — concerning the quality, safety, or conduct of clinical care — which are routed to the Clinical Lead / Clinical Advisory Function; and
- **Platform complaints** — concerning the technology, billing, scheduling, or service — handled by Faresay's support/operations function.

12.2 Clinical complaints will be acknowledged within [PLACEHOLDER: timeframe] and investigated by, or under the oversight of, the Clinical Lead, with the Practitioner given a fair opportunity to respond. Outcomes, learning, and any actions (including referral to fitness-to-practise processes where warranted) will be recorded and fed into incident management (Section 11) and quality assurance (Section 10).

12.3 Clients retain the right to complain directly to the Practitioner's Professional Body / licensing board, and Faresay will not obstruct this. Faresay will signpost the relevant body where appropriate. [PLACEHOLDER: complaints contact / address.]

12.4 ⚠️ COUNSEL: Confirm any regulatory complaint-handling requirements, timeframes, and record-keeping obligations applicable in the UK and relevant US States, and how clinical complaints interact with Faresay's terms (`terms-of-service.md`).

13. Suitability / Exclusion Criteria for Onboarding Clients

13.1 The Platform is suitable for adults (and, only if and where Faresay decides to serve them, minors — see below) seeking telehealth mental-health care for presentations that can be safely treated remotely on a cash-pay, planned-care basis.

13.2 ⚠️ CLINICAL: Client suitability screening and exclusion criteria must be defined by the clinical lead and aligned with Section 6 and the Crisis / Safeguarding Policy. Indicative exclusion/triage criteria for onboarding a Client:

- individuals in acute crisis or at imminent risk of harm to self or others (route to emergency services / crisis pathways per `crisis-safeguarding-policy.md`);
- presentations requiring emergency, in-person, or higher-intensity care than telehealth can safely provide (see 6.2);
- individuals located outside a jurisdiction where a suitably-licensed Practitioner is available;
- **minors** — [PLACEHOLDER: whether under-18s are served; if so, age thresholds, parental/guardian consent, and additional safeguards]. ⚠️ CLINICAL / ⚠️ COUNSEL: serving minors materially increases clinical and legal complexity (consent, safeguarding, mandatory reporting); confirm explicitly whether the Platform serves minors at launch.

13.3 Where a prospective or current Client is identified as unsuitable, Faresay and/or the Practitioner will signpost to more appropriate services and follow the escalation steps in the Crisis / Safeguarding Policy where risk is present.

14. Audit and Periodic Review

14.1 Faresay will periodically audit compliance with this Policy, including: a sample audit of Practitioner verification and ongoing-monitoring records (Section 4); review of incident and complaint logs and actions (Sections 11–12); and review of quality-assurance outputs (Section 10).

14.2 This Policy will be reviewed at least [PLACEHOLDER: e.g. annually], and additionally on any material change to law, regulation, professional standards, the business model, or following a serious incident. Review is led by the Clinical Lead and approved by [PLACEHOLDER: approving body].

14.3 Material clinical risks identified through audit and review are recorded in the risk register (`risk-register.md`).

15. Roles and Responsibilities Matrix

⚠ CLINICAL / ⚠ COUNSEL: Indicative only. The allocation below — particularly which responsibilities sit with the MSO vs the professional entity ("friendly PC") in US corporate-practice states — must be confirmed with the clinical lead and counsel. [PLACEHOLDER: named roles/owners.]

Area	Practitioner	Clinical Lead	Clinical Advisory Function	Faresay (non-clinical: trust & safety / ops / engineering)
Clinical decisions for individual clients	Owns / accountable	— (no individual-case direction)	—	—
This Policy & clinical standards	Comply	Owns / maintains	Advises / endorses	Supports operationally
Onboarding & verification (Section 4)	Provide evidence	Sets criteria; approves exceptions	Advises	Runs checks operationally
Ongoing licence/sanction monitoring (4.7)	Keep credentials current; disclose	Reviews flags; suspends eligibility	Advises	Runs monitoring; suspends availability operationally
Scope of practice / exclusions (Sections 6, 13)	Works within competence	Defines criteria	Endorses	Implements platform controls
Informed consent (Section 7)	Obtains & documents	Sets standard	Advises	Provides supporting tooling
Clinical records (Section 8)	Owns / holds / retains	Sets minimum standards	Advises	Provides secure platform tooling only
Supervision & CPD (Section 9)	Arranges & evidences	Sets minimum requirement	Advises	Verifies evidence
Quality assurance & outcomes (Section 10)	Participate	Owns programme	Conducts case review	Provides data tooling
Incidents (Section 11)	Report; act on client safety	Reviews / oversees	Reviews serious incidents	Logs; triages

Area	Practitioner	Clinical Lead	Clinical Advisory Function	Faresay (non-clinical: trust & safety / ops / engineering)
Complaints — clinical (Section 12)	Respond	Oversees clinical route	Reviews	Triages; handles platform complaints
Audit & review (Section 14)	Cooperate	Leads	Participates	Supports

16. References to Professional and Ethical Frameworks

16.1 ⚠️ CLINICAL / ⚠️ COUNSEL: The lists below are **indicative** and must be confirmed and completed by the clinical lead and counsel — including which bodies/registers Faresay will recognise, and the correct, current standards for each discipline and jurisdiction.

16.2 United Kingdom (indicative).

- **HCPC** (Health and Care Professions Council) — statutory regulator of practitioner psychologists (and others).
- **BACP** (British Association for Counselling and Psychotherapy) — ethical framework and accreditation for counsellors/psychotherapists.
- **UKCP** (UK Council for Psychotherapy) — register and standards for psychotherapists.
- **BPS** (British Psychological Society) — professional body / practice guidance for psychologists.
- [PLACEHOLDER: other relevant bodies — e.g. BABCP, NCPS — and which Faresay recognises.]

16.3 United States (indicative).

- **State licensing boards** — the primary source of licensure authority and disciplinary rules in each State (psychology, professional counselling, clinical social work, marriage & family therapy).
- **APA** (American Psychological Association) — Ethical Principles of Psychologists and Code of Conduct (psychologists).
- **ACA** (American Counseling Association) — Code of Ethics (counsellors).
- **NASW** (National Association of Social Workers) — Code of Ethics (clinical social workers).
- [PLACEHOLDER: other relevant codes/bodies — e.g. AAMFT for marriage & family therapists.]
- Interstate compacts — **PSYPACT**, the **Counseling Compact**, the **Social Work Licensure Compact** (see Section 4).

16.4 Practitioners remain bound by the ethical and practice standards of the body that registers/licenses them, in addition to this Policy. Where this Policy and a Practitioner's professional standards conflict, the Practitioner must comply with their professional/legal obligations and raise the conflict with the Clinical Lead.

17. Related Documents

- `CONTEXT.md` — shared fact sheet (source of truth).
- `therapist-agreement.md` — Practitioner contractual terms (credentialing, records, fee characterisation).

- `crisis-safeguarding-policy.md` — crisis, risk, safeguarding, and escalation pathways (cross-referenced throughout).
 - `privacy-policy.md` and `security-data-protection-policy.md` — data protection and security.
 - `terms-of-service.md` — client-facing terms.
 - `risk-register.md` — risk capture and tracking.
-

End of DRAFT v0.1. ⚠️ This document must be validated by a qualified clinical lead (and, for the points flagged ⚠️ COUNSEL, by counsel) before use.

Crisis & Safeguarding Policy

⚠️ DRAFT v0.1 — for clinical advisor + legal review. NOT clinical or legal advice. Must be validated by a qualified clinical/safeguarding lead before use. Last updated: 26 June 2026

Faresay — Crisis & Safeguarding Policy

This policy sets out how Faresay and the Therapists who use the Faresay marketplace platform (the "**Platform**") identify, respond to, escalate, and learn from situations involving risk to a Client's safety or the safety of others, and how Faresay discharges its safeguarding responsibilities.

It must be read alongside the [Clinical Governance Policy](#), the [Terms of Service](#), the [Privacy Policy](#), the [Therapist Agreement](#), and the [Security & Data Protection Policy](#). Where this policy and another Faresay document conflict on a safety-critical point, the more protective interpretation applies pending resolution by the Safeguarding Lead.

⚠️ CLINICAL — This document is a structural first draft. Every clinical threshold, screening criterion, escalation step, and exclusion criterion below must be reviewed, corrected, and signed off by a qualified clinical/safeguarding lead before any operational use. Do not rely on it as written.

1. Purpose & Scope

1.1 Purpose. This policy aims to: - (a) protect Clients, Therapists, third parties, and the public from foreseeable serious harm; - (b) make clear that Faresay is **not** an emergency or crisis service, and ensure that message is communicated consistently; - (c) give Therapists a clear, predictable framework for identifying and escalating risk; - (d) define safeguarding roles, responsibilities, and on-call expectations within Faresay; and - (e) ensure incidents are documented, reviewed, and used for learning, consistent with legal and professional obligations.

1.2 Who this applies to. This policy applies to: - all Therapists providing Clinical Services via the Platform (it is incorporated into the [Therapist Agreement](#) by reference — see clauses 6.6–6.7 of that agreement); - all Faresay staff and contractors involved in operations, trust & safety, support, or clinical governance; and - the design and operation of Platform features that touch on safety.

1.3 Territorial scope. Faresay currently operates in the **United Kingdom** and plans to expand into the **United States (all 50 states + DC)** on a cash-pay / out-of-network model (see [CONTEXT.md](#)). This policy covers UK obligations now and forward-looking US obligations for expansion. US-specific provisions are forward-looking and must be finalised before any US launch. ⚠️ COUNSEL / ⚠️ CLINICAL — Many obligations below vary **by US state** and **by UK nation** (England, Scotland, Wales, Northern Ireland); jurisdiction-specific addenda will be required.

1.4 Population scope. This draft assumes the Platform serves **adults only** and that minors are out of scope, consistent with the current assumption in the [Terms of Service](#) and [Therapist Agreement](#). ⚠️ CLINICAL / ⚠️ COUNSEL — If services to minors are ever offered, this policy requires substantial additional child-safeguarding content (consent models, age thresholds — which vary by US state and within the UK, e.g. Gillick/Fraser considerations — verification, and dedicated child-protection escalation). The "safeguarding of children" provisions below currently address (a) risk disclosed about a child who is a third party (e.g. a Client's own child) and (b) future-proofing, not direct treatment of minors.

1.5 Relationship to clinical responsibility. Faresay provides the **technology and marketing platform**; the **Therapist** provides the clinical service and owns the clinical relationship, clinical judgement, and clinical record

(see [CONTEXT.md](#) and [Terms of Service §2](#)). Nothing in this policy transfers clinical responsibility for an individual Client to Faresay or makes Faresay a healthcare provider. The Therapist remains the decision-maker on the clinical care of their Clients, within the framework this policy sets.

2. Faresay Is Not an Emergency or Crisis Service

2.1 Core statement. Faresay and the Platform are **not** an emergency service, crisis line, or suicide-prevention service. Bookings, messages, and other communications sent through the Platform are **not monitored in real time for emergencies**, and Therapists are not available for emergency, urgent, or crisis response. The Platform is intended for **non-urgent, scheduled** mental-health support.

2.2 How this is communicated to Clients. This message must be surfaced clearly and repeatedly, not buried. At minimum (⚠️ CLINICAL / ⚠️ COUNSEL — confirm placement, wording, and frequency): - (a) in the [Terms of Service](#) (see ToS §3, "NOT AN EMERGENCY OR CRISIS SERVICE"), which Clients accept at onboarding; - (b) on the onboarding / intake flow, as a distinct, acknowledged screen — not a buried clause; - (c) persistently within the messaging interface (e.g. a standing banner or footer stating the Platform is not monitored for emergencies and listing crisis resources — see §8); - (d) within any "report a concern" or help surfaces; and - (e) reiterated by the Therapist at the start of the therapeutic relationship as part of informed consent (see [Clinical Governance Policy](#)).

2.3 Wording must be consistent. The crisis-resource wording and numbers used across the Platform, the Terms of Service, and this policy must match. The authoritative list of resources is in §8 of this policy. ⚠️ COUNSEL / ⚠️ CLINICAL — Verify all numbers and service descriptions and keep them synchronised with [Terms of Service §3](#).

2.4 What "not an emergency service" does not mean. This disclaimer does **not** relieve a Therapist of their professional duty of care to a Client during the course of a session or active treatment relationship (see §4), nor of mandatory-reporting and duty-to-warn obligations (see §6–§7). It governs Client expectations of the Platform's availability and monitoring; it does not switch off a clinician's professional and legal duties.

3. Risk Types Covered

This policy covers the following risk presentations. ⚠️ CLINICAL — definitions, indicators, and thresholds for each must be set by the Safeguarding Lead; the descriptions below are placeholders for scope, not clinical guidance.

3.1 Suicidality — suicidal ideation (passive or active), intent, plan, means, and history; acute and chronic risk.

3.2 Self-harm — non-suicidal self-injury and related risk, including escalation patterns and medical risk.

3.3 Risk of harm to others — threats or risk of violence toward identifiable or non-identifiable third parties, including homicidal ideation. Engages possible **duty-to-warn / duty-to-protect** obligations (see §7).

3.4 Domestic abuse — a Client experiencing or perpetrating domestic abuse (physical, sexual, psychological, coercive control, economic, or technology-facilitated abuse). ⚠️ CLINICAL / ⚠️ COUNSEL — Special caution: documentation, safety planning, and any external contact must not increase risk to a Client experiencing abuse; covert tech-facilitated monitoring of the victim must be assumed possible.

3.5 Safeguarding of children — concern that a child (typically a third party, e.g. a Client's child, in this adults-only model) is suffering or at risk of significant harm, abuse, or neglect. Engages **mandatory / statutory reporting** considerations (see §6).

3.6 Safeguarding of vulnerable / at-risk adults — concern that an adult with care-and-support needs is experiencing or at risk of abuse or neglect (sometimes "adult safeguarding"). Engages jurisdiction-specific adult-safeguarding and reporting frameworks (see §6).

3.7 Other acute risk — e.g. acute psychosis, severe disordered eating with medical risk, acute intoxication/overdose risk, severe deterioration. May indicate the presentation is **unsuitable for the Platform** (see

§5 exclusion criteria).

4. Therapist Duty of Care & Responsibilities

4.1 Professional duty. Each Therapist remains bound by the duty of care, ethical codes, and standards of their regulator and professional body (e.g. in the UK: HCPC, BACP, UKCP, BPS, Social Work England / equivalents; in the US: state licensing boards and the relevant professional code). ⚠️ CLINICAL — confirm the applicable bodies and standards per jurisdiction.

4.2 Therapists must: - (a) screen for and remain alert to the risk types in §3 at onboarding and on an ongoing basis (see §5); - (b) hold and maintain competence in risk assessment and management appropriate to the populations they serve, and complete Faresay's required training (see §11); - (c) follow the escalation protocol in §5–§6 when risk is identified; - (d) comply with all mandatory-reporting, duty-to-warn/protect, and safeguarding obligations applicable in the jurisdiction where the Client is located (see [Therapist Agreement §6.6](#)); - (e) maintain confidentiality, disclosing only as permitted or required by law or to protect against serious harm (see §7 and [Therapist Agreement §6.5](#)); - (f) document risk, concerns, decisions, and actions contemporaneously in the clinical record (see §9); - (g) carry their own professional indemnity / malpractice insurance as required by the [Therapist Agreement](#); and - (h) not take on, and promptly escalate/offboard, Clients whose presentation falls within the exclusion criteria (§5).

4.3 Scope of the Therapist's role in crisis. The Therapist is responsible for clinical risk decisions for their own Clients. The Therapist is **not** expected to provide emergency response in place of statutory emergency services; the appropriate action in an acute emergency is to direct the Client to emergency services and, where duties require and it is safe and lawful, to make contact with emergency or relevant authorities (see §5).

4.4 Availability and limits. Therapists must make their availability and response-time limits clear to Clients (the Platform is not monitored for emergencies; messages are not answered in real time). ⚠️ CLINICAL — define expected message-response windows and how Therapists communicate their boundaries.

5. Client Risk Screening, Ongoing Assessment & Exclusion Criteria

5.1 Screening at onboarding. Before or at the start of the therapeutic relationship, risk-relevant information should be gathered to determine whether the Platform and the matched Therapist are an appropriate setting for the Client. ⚠️ CLINICAL — The Safeguarding/Clinical Lead must define the screening instrument(s), questions, scoring, and decision rules. At minimum, screening should consider current and historical suicidality/self-harm, risk to others, acute psychiatric presentation, substance use, and the Client's location (for emergency routing and licensure — see §8.3 and [CONTEXT.md](#)).

5.2 Ongoing assessment. Risk is dynamic. Therapists must reassess risk throughout the relationship, in particular at transitions (intake, deterioration, major life events, medication changes, ending of therapy) and whenever risk indicators appear in session or in messages.

5.3 Exclusion criteria — presentations unsuitable for cash-pay telehealth. Certain presentations are **not** suitable for delivery via an unscheduled-monitoring, remote, cash-pay marketplace and should be excluded or referred to a higher level of care. ⚠️ CLINICAL — the Clinical/Safeguarding Lead must finalise this list; this cross-references and must remain consistent with the suitability/exclusion provisions in the [Clinical Governance Policy](#). Indicative (placeholder) examples to be confirmed: - (a) acute, high, or imminent risk of suicide or serious self-harm requiring crisis or inpatient care; - (b) current intent or plan to seriously harm another person; - (c) acute psychosis, mania, or severe decompensation requiring urgent/specialist or in-person care; - (d) severe substance dependence requiring medically supervised detox/withdrawal; - (e) severe eating disorders with medical instability; - (f) presentations requiring a level of monitoring, frequency, or coordination the Platform cannot safely provide; - (g) [PLACEHOLDER: other criteria to be defined by the Clinical/Safeguarding Lead].

5.4 Action where exclusion criteria apply. Where a Client's presentation meets an exclusion criterion, the Therapist must not continue routine treatment as if the setting were appropriate; instead they must (in line with clinical judgement and §6 escalation): support the Client to access an appropriate level of care, signpost crisis/emergency resources (§8), document the decision (§9), and notify Faresay's Safeguarding Lead where required. ⚠️ CLINICAL / ⚠️ COUNSEL — abandonment risk: ending or declining care must be handled to avoid clinical abandonment and to meet professional/ethical "termination of care" rules in the jurisdiction (coordinate with [Clinical Governance Policy](#) and [Therapist Agreement](#) continuity-of-care provisions).

5.5 Matching and licensure gate. A Therapist must be licensed in the jurisdiction where the Client is located (UK; and in the US, the relevant state — leveraging PSYPACT / Counseling Compact / Social Work Licensure Compact per [CONTEXT.md](#)). Location is also needed for emergency routing (§8.3). ⚠️ COUNSEL — confirm licensure verification gates by jurisdiction.

6. Crisis Identification & Step-by-Step Escalation Protocol

⚠️ CLINICAL — This protocol is a **structural placeholder**. The Clinical/Safeguarding Lead must define the actual clinical decision thresholds, who does what, timeframes, and jurisdiction-specific contacts before any operational use. The numbered steps below describe the *shape* of the process, not validated clinical instructions.

6.1 Identify. A Therapist (or, where a Platform safety signal is triggered, Faresay's Trust & Safety function) identifies indicators of acute risk (§3) — disclosed directly, observed in session, or surfaced via a risk flag (§8.2).

6.2 Assess severity and immediacy. The Therapist assesses whether the risk is: - **Imminent / life-threatening** (e.g. active attempt in progress, immediate intent and means, immediate danger to a child or another person); or - **Elevated but not immediately life-threatening**; or - **Lower-level / chronic** but requiring monitoring and planning. ⚠️ CLINICAL — define indicators and thresholds for each tier.

6.3 Immediate-danger pathway (imminent/life-threatening). - (a) Prioritise getting the Client/affected person to emergency help **now**: direct them to call emergency services (US **911**; UK **999**) or, where safe and within the Therapist's lawful authority and professional duty, contact emergency services on their behalf using the Client's location (see §8.3). - (b) Surface and provide crisis resources (§8). - (c) Where duty-to-warn/protect or mandatory-reporting obligations are engaged, follow §6.5 / §7 / §6.6. - (d) Stay with / maintain contact to the extent clinically appropriate and possible within the session. - (e) Notify Faresay's Safeguarding Lead / on-call per §6.7 and document per §9. ⚠️ CLINICAL / ⚠️ COUNSEL — Contacting emergency services without consent, and using location data to do so, has clinical, legal, and confidentiality implications that vary by jurisdiction; confirm lawful basis (and see the emergency data-sharing carve-out, §8.4).

6.4 Elevated-risk pathway (not immediately life-threatening). - (a) Conduct/complete a structured risk assessment and collaborative safety plan with the Client. - (b) Signpost crisis resources (§8) and agree what the Client will do if risk escalates. - (c) Consider increased session frequency, coordination with the Client's GP/primary care or other providers (with consent where required), and whether exclusion criteria (§5.3) now apply. - (d) Document and, where thresholds are met, escalate to the Safeguarding Lead (§6.7) and follow reporting duties (§6.6/§7).

6.5 Duty-to-warn / duty-to-protect trigger (risk to others). Where a Client presents a serious risk of harm to an identifiable third party, the Therapist must follow the duty-to-warn / duty-to-protect framework applicable in the Client's jurisdiction (see §7). ⚠️ COUNSEL — this is **state-variable in the US** and threshold-based in the UK.

6.6 Mandatory-reporting trigger (children / vulnerable adults). Where information indicates a child or vulnerable/at-risk adult is suffering or at risk of significant harm, the Therapist must follow the mandatory/statutory reporting framework applicable in the Client's jurisdiction (see §6 reporting detail in §7) and notify the

Safeguarding Lead. ⚠️ COUNSEL / ⚠️ CLINICAL — reporting duties, recipients, and timeframes vary by US state and UK nation.

6.7 Internal escalation to Faresay. In parallel with clinical action, the Therapist notifies Faresay's **Safeguarding Lead / on-call contact** of a serious incident via [PLACEHOLDER: defined reporting channel — e.g. dedicated email, phone, in-app report], within [PLACEHOLDER: timeframe]. Faresay's role here is **supportive and governance-focused** (logging, support, pattern detection, learning, regulatory/notification obligations) — not to override the Therapist's clinical decisions for an individual Client. ⚠️ CLINICAL — define the threshold for mandatory internal escalation, the channel, and the timeframe.

6.8 Platform safety signals. Where a Platform feature (e.g. a self-harm keyword flag, §8.2) raises a signal independent of a Therapist, define how Trust & Safety triages it, what is automated vs human-reviewed, and how false positives and Client privacy are handled. ⚠️ CLINICAL / ⚠️ COUNSEL — automated risk detection on sensitive mental-health content carries clinical, false-negative/positive, privacy, and liability risks; confirm before building.

7. Confidentiality Breach Thresholds, Duty-to-Warn & Mandatory Reporting

7.1 General principle. Client confidentiality is a core duty (see [Therapist Agreement §6.5](#), [Privacy Policy](#)). It may be **lawfully overridden only** where permitted or required to prevent or reduce a risk of serious harm, or where the law requires disclosure. Disclosure should be the **minimum necessary** to address the risk, made to the appropriate recipient, and documented (§9).

7.2 United States — duty to warn / duty to protect (Tarasoff line of authority). ⚠️ COUNSEL — **State-variable.** Following *Tarasoff* and its progeny, many US states impose a duty to warn and/or protect identifiable third parties from a Client's serious threats; some states make it mandatory, some permissive, some have no statute, and the trigger, the discharge mechanism (warn the victim, notify police, initiate hospitalisation, etc.), and the immunity provisions differ **state by state**. The applicable rule is generally that of the state where the Client/Therapist is situated. Counsel must produce a **per-state matrix** of duty-to-warn/protect obligations and safe-harbour steps before US launch, and the Platform/training must reflect each state's rule.

7.3 United Kingdom — confidentiality-breach thresholds. ⚠️ COUNSEL / ⚠️ CLINICAL — UK practitioners may breach confidentiality to prevent serious harm, broadly under common-law public-interest disclosure, data-protection conditions for processing special-category data, and professional-body guidance (e.g. GMC/BACP/HPC-aligned principles, ICO guidance). Counsel/clinical lead must define the threshold ("real and serious risk of serious harm"), who may be told (e.g. police, the at-risk person, GP), and how to record the justification — and note variation across England, Scotland, Wales, and Northern Ireland.

7.4 Mandatory reporting — children. ⚠️ COUNSEL / ⚠️ CLINICAL — In the US, child-abuse mandatory-reporting laws apply to therapists in **all states but differ by state** (who must report, what triggers it, to whom, and timeframes). In the UK, the framework is statutory and nation-specific (e.g. local-authority children's services / multi-agency safeguarding hubs; police; relevant working-together guidance). Counsel/clinical lead must specify the recipient, trigger, and timeframe for each jurisdiction.

7.5 Mandatory / permitted reporting — vulnerable & at-risk adults. ⚠️ COUNSEL / ⚠️ CLINICAL — US adult-protective-services reporting duties vary by state (some mandatory for certain professionals, some permissive). In the UK, adult-safeguarding duties run through local-authority adult social care (e.g. Care Act 2014 in England and equivalents in the other nations). Specify recipients, triggers, and timeframes per jurisdiction.

7.6 Domestic abuse. ⚠️ COUNSEL / ⚠️ CLINICAL — Reporting and information-sharing in domestic-abuse contexts is sensitive and jurisdiction-specific; disclosure can increase danger to a victim. Define safe practice, lawful basis, and any mandatory elements per jurisdiction; do not contact a victim's household or share information in ways that could be intercepted by an abuser.

7.7 **Conflict of duties.** Where the obligations of the Client's jurisdiction, the Therapist's licence, professional ethics, and this policy appear to conflict, the Therapist should apply the most protective lawful course and seek guidance from the Safeguarding Lead and, where time permits, their own professional/legal advisers. ⚠️ COUNSEL.

8. Emergency Resources & Safety Features

8.1 **Crisis resources to surface.** The following resources must be displayed in the surfaces described in §2.2. ⚠️ COUNSEL / ⚠️ CLINICAL — **Verify, localise, and keep synchronised with Terms of Service §3.**

United States - 988 Suicide & Crisis Lifeline — call or text **988**. - **911** — for any life-threatening emergency. - [PLACEHOLDER: state/territory-specific or population-specific crisis resources to be added per jurisdiction.]

United Kingdom - 999 — for any life-threatening emergency. - **111** — for urgent (non-life-threatening) NHS help. - **Samaritans** — free on **116 123** (24 hours a day). - [PLACEHOLDER: nation-specific resources / text services to be added — e.g. text-line services; confirm before publication.]

If the Client is outside these countries, direct them to their local emergency services. ⚠️ CLINICAL — define handling for Clients located outside supported jurisdictions.

8.2 **Risk flags.** The Platform may provide features to flag risk (e.g. a Therapist-set risk indicator on a Client record, or detection signals on messages). ⚠️ CLINICAL / ⚠️ COUNSEL — Define what flags exist, who sets/sees them, how they affect care and matching, how privacy is protected, and the risk of automated detection (false positives/negatives). Any automated scanning of clinical/message content must have a confirmed lawful basis and be assessed for clinical safety before deployment.

8.3 **Location capture for emergencies.** To enable emergency routing (correct emergency number; ability to direct or send help to the right place; correct duty-to-warn/reporting jurisdiction), the Platform should capture the Client's **location/jurisdiction**. ⚠️ COUNSEL / ⚠️ CLINICAL — Confirm what location data is collected (self-declared jurisdiction vs precise geolocation), when, lawful basis, retention, and how it is used in an emergency; precise real-time geolocation has significant privacy implications and must be assessed in the DPIA / privacy review (see [Privacy Policy](#) and [Security & Data Protection Policy](#)).

8.4 **Emergency data-sharing carve-out.** In a life-at-risk emergency, Faresay and/or the Therapist may need to share limited personal data (e.g. identity, location, contact details, nature of risk) with emergency services or relevant authorities **without prior consent** to protect the vital interests of the Client or another person. This carve-out must be reflected in, and operated consistently with, the [Privacy Policy](#) and the [Security & Data Protection Policy](#). ⚠️ COUNSEL — Confirm the lawful basis (e.g. UK GDPR vital-interests / public-interest / legal-obligation conditions for special-category data; and, in the US, HIPAA's permitted disclosures to avert a serious and imminent threat and state-law equivalents), the minimum-necessary principle, and logging requirements.

9. Documentation & Record of Concerns

9.1 **Clinical record.** The Therapist must contemporaneously document risk assessments, safety plans, disclosures, decisions (including the reasoning for any confidentiality breach or report), actions taken, and who was contacted, in the **clinical record** they own (see [CONTEXT.md](#), [Therapist Agreement](#)). ⚠️ CLINICAL — define minimum documentation standards and templates.

9.2 **Faresay record of concerns.** Faresay should maintain a secure **safeguarding incident log / record of concerns** capturing reported serious incidents, escalations, and outcomes, accessible only to authorised safeguarding/governance personnel. ⚠️ CLINICAL / ⚠️ COUNSEL — define what Faresay records vs what stays in

the clinician's clinical record (to respect the controller/processor split in [Privacy Policy §1](#)), access controls, and retention.

9.3 Retention & security. Records of concern are highly sensitive special-category data and must be retained and secured per the [Privacy Policy](#) and [Security & Data Protection Policy](#), for the retention period set there. ⚠️
COUNSEL — confirm retention periods (UK + US state law).

9.4 Auditability. Escalations, reports, and emergency data-sharing decisions should be time-stamped and auditable to support post-incident review (§10) and any regulatory inquiry.

10. Safeguarding Leads, Roles & On-Call Expectations

10.1 Safeguarding Lead. Faresay will designate a **Safeguarding Lead** (and a **Deputy**) with defined responsibilities for this policy, incident oversight, escalation support, liaison with authorities where appropriate, and learning. ⚠️
CLINICAL — The Safeguarding Lead should be (or be advised by) a suitably qualified clinical/safeguarding professional. Confirm the required qualification, the appointment, and named contacts. – **Safeguarding Lead:** [PLACEHOLDER: name / role / contact] – **Deputy Safeguarding Lead:** [PLACEHOLDER: name / role / contact] – **Clinical Lead / Advisor:** [PLACEHOLDER: name / role / contact] (see [Clinical Governance Policy](#))

10.2 Trust & Safety function. [PLACEHOLDER: team/role] triages Platform safety signals and non-clinical reports and escalates to the Safeguarding Lead per §6.8.

10.3 On-call expectations. ⚠️ CLINICAL / ⚠️ COUNSEL — Decide and document whether Faresay offers any out-of-hours safeguarding contact, the response-time expectation, and the limits of that contact. **Critically:** an on-call channel must **not** be presented to Clients as an emergency or crisis line — doing so would contradict §2 and could create reliance and liability. Any internal on-call is for Therapist/staff escalation and governance, not Client emergency response. Confirm scope, staffing, and SLAs.

10.4 Limits of Faresay's role. Faresay does not provide clinical care, does not supervise the clinical judgement of independent Therapists, and is not a substitute for emergency services. Its safeguarding role is governance, support, logging, learning, and meeting its own legal/regulatory obligations.

11. Therapist Training Requirements

11.1 Required training. Before onboarding and periodically thereafter, Therapists must complete and attest to Faresay's required training on this policy, covering: identifying and assessing the risk types in §3; the escalation protocol (§6); confidentiality-breach thresholds, duty-to-warn/protect, and mandatory reporting in the jurisdiction(s) they serve (§7); use of Platform safety features (§8); and documentation (§9). ⚠️ CLINICAL — define the curriculum, format, assessment, and refresh cadence.

11.2 Jurisdiction-specific knowledge. Therapists must be familiar with the duty-to-warn/protect and mandatory-reporting rules of every jurisdiction in which they hold themselves out to treat Clients. ⚠️ COUNSEL / ⚠️ CLINICAL — Faresay should provide a per-jurisdiction reference (the §7.2 matrix) but the Therapist remains professionally responsible.

11.3 Competence. Therapists must only accept Clients within their competence and must keep risk-management skills current per their professional body's CPD requirements. ⚠️ CLINICAL.

11.4 Records. Faresay should record training completion and attestations as part of credentialing (see [Clinical Governance Policy](#)).

12. Post-Incident Review & Learning

12.1 **Review of serious incidents.** Serious safeguarding/crisis incidents should be reviewed by the Safeguarding Lead (with the Clinical Lead and, where appropriate, the Therapist) to identify what happened, whether this policy and the escalation protocol were followed, what worked, and what should change. ⚠️ CLINICAL — define what counts as a "serious incident" requiring review, and the review method and timeframe.

12.2 **Learning loop.** Findings should feed back into training (§11), this policy, the [Clinical Governance Policy](#), screening/exclusion criteria (§5), and Platform safety features (§8). Themes and trends should be reviewed periodically.

12.3 **Support & a just culture.** Reviews should support a just, learning-oriented culture (supporting Clients, Therapists, and staff affected by a serious incident) rather than being purely punitive, while preserving accountability where standards were not met. ⚠️ CLINICAL.

12.4 **External duties.** ⚠️ COUNSEL / ⚠️ CLINICAL — Confirm any external reporting/notification duties arising from a serious incident (e.g. to regulators, professional bodies, data-protection authorities for any associated breach, or coroner/medical-examiner processes), per jurisdiction.

13. Review Cadence

13.1 This policy will be reviewed at least **annually**, and additionally after any serious incident (§12), after any material change in law or professional guidance, and before entering a new jurisdiction. ⚠️ CLINICAL / ⚠️ COUNSEL — confirm cadence and owner.

13.2 **Owner:** [PLACEHOLDER: Safeguarding Lead]. **Next review due:** 26 June 2026.

13.3 **Version control.** Material changes should be versioned and the "Last updated" date refreshed. This is **DRAFT v0.1** and must not be operationalised until validated by a qualified clinical/safeguarding lead and reviewed by counsel.

⚠️ DRAFT v0.1 — Not for operational use. Validate with a qualified clinical/safeguarding lead (⚠️ CLINICAL) and legal counsel (⚠️ COUNSEL) before publication or reliance. Keep crisis resources synchronised with the [Terms of Service](#).